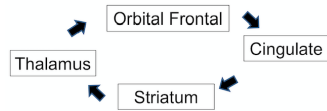


PSYCH DISORDERS

Category	Disorder	Diagnostic criteria/symptoms	Time Frame	Treatments	Notes
Anxiety Disorders All share in common the theme of inappropriate anxiety. Anxiety is abnormal when it is elicited by inappropriate cues (or no cues), excessive in intensity, excessive in duration (usually >6 months)	Panic Disorder	- recurrent and unexpected panic attacks - fear of future panic attacks OR - a significant change in behavior related to the attacks (avoidance of exertion) - episodes occur in random (untriggered, unpredictable)	> 1 mo	Cognitive Behavioral Therapy Anxiolytic Medications (antidepressants, benzodiazepines)	Panic Attack: Intense fear with somatic or cognitive symptoms such as: - Cardiac symptoms - Sweating - Trembling - Shortness of breath - Choking - Chest pain - GI symptoms - Feeling faint - Feeling detached - Fear of losing control - Fear of dying - Tingling/numbness - Chills/hot flashes Abrupt onset with episode peak within minutes
	Agoraphobia	More than two of the following situations are feared/avoided - using public transportation - being in open spaces - being in enclosed spaces - standing in line or being in a crowd - being outside of home alone			These situations are feared/avoided because of thoughts that escape might be difficult (or might not be available) if incapacitating/embarrassing symptoms develop Agoraphobia often develops in Panic Disorder (PD pt avoids multiple situations due to fear of not getting help during a panic attack). But it can develop without PD as well.
	Specific Phobia	Excessive, persistent, and disproportional fear of an object or situation. The fear invariably occurs when exposed to the stimulus, or the person avoids the phobic situation. The fear causes marked distress or.			Subtypes: - animal - natural environment - situational - blood-injection-injury - other

		Impairment and isn't accounted for by a different disorder.			
	Social Anxiety Disorder (formerly "social phobia")	- Excessive persistent and unrealistic fear of social situations involving possible scrutiny by others due to a fear of negative evaluation - As a result, social situations are avoided or endured with great distress			Specify "performance only" if the fear is restricted to performing in public (ie. Public speaking, athletics) Major differential: avoidant personality
	Generalized Anxiety Disorder (GAD)	- Persistent, uncontrolled anxiety about multiple events Accompanied by symptoms such as - Restless, on edge - Decreased concentration - Muscle tension - Fatigued - Irritability - insomnia	> 6 mos		
	Separation anxiety disorder	- developmentally inappropriate and excessive anxiety concerning separation from major attachment figure(s) Person experiences marked distress (both mental and physical) about: - anticipated or actual separation - losing or harm befalling major attachment figure or self that leads to separation - going out (to school/work/sleeping away) because of separation fears			
	Selective Mutism	- Refusal to speak in specific situations despite fluent speech in other contexts - Written communication and social play may also be affected - Refusal to speak is anxiety-based	Symptoms persist for less than 1 month (not the first month of school)		Not due to lack of language knowledge or a communication disorder such as stuttering

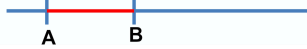
Obsessive-Compulsive and Related Disorders	Obsessive-Compulsive Disorder (OCD)	- Recurrent obsessions (intrusive and recurrent thoughts/urges/images/ that increase distress) and/r compulsions (repetitive behaviors/mental acts performed to decrease distress) – both obsession and compulsion don't need to be present; one is sufficient for dx - Obsessions/compulsions are time consuming or distressful/disruptive - Symptoms are not explained by another disorder (ie. Weight preoccupation in eating disorders)		Behavioral Therapy (exposure and response prevention – ERP) Medications (SSRIs) Treatment options for Refractory OCD: Psychosurgery: surgical (or radiation) lesions of either the anterior cingulate gyrus (cingulotomy) or anterior limb of the internal capsule (capsulotomy) Deep Brain Stimulation: electrical impulses delivered by indwelling brain electrode attached to an implanted thoracic pacemaker	Compulsions aren't always logically related to the obsession. Neuroanatomical Correlates: -Cortico-striato-thalamo-cortical (CSTC) circuit <i>overactivity</i>  <pre> graph TD OF[Orbital Frontal] --> C[Cingulate] C --> S[Striatum] S --> T[Thalamus] T --> OF </pre> -Can also be caused by serotonin deficiency (on the basis that SSRIs help improve symptoms) Some people are “with absent insight” meaning they’re delusional and believe their obsessions & compulsions are normal.
	Body Dysmorphic Disorder	- Obsessive Compulsive behavior about perceived defect in appearance (<i>flaw is minimal/non-observable/trivial</i>) Repetitive behaviors or mental acts are performed in response to the appearance concerns - The preoccupation is not better accounted for by an eating disorder - The preoccupation in BDD must cause functional impairment (ie. Sequestering oneself) – otherwise, the preoccupation may be considered normal vanity.			The belief can be of delusional intensity (100% certainty) Add specifier BDD “with absent insight (delusional beliefs)”
	Hoarding	- Obsessive-compulsive behavior resulting in the excessive accumulation of possessions - The compulsion to hoard			Behaviors are not better explained by another disorder

Somatic Symptom and Related Disorders: The common feature of these disorders is the prominence of somatic/health-related symptoms associated with significant distress or impairment.	Excoriation	Recurrent, unwanted skin picking causing lesions			Behaviors are not better explained by another disorder
	Trichotillomania	Recurrent, unwanted pulling out of one's hair			Behaviors are not better explained by another disorder
	Somatic Symptom Disorder (SSD) Illness Anxiety Disorder	- More than 1 distressing/disruptive somatic symptom - At least one indicator of excessive thoughts/feelings/behaviors about the symptom(s) such as: - Disproportionate thoughts about the seriousness of the symptom - High levels of anxiety about the symptom or health - Excessive time/energy devoted to the symptom			- The diagnosis of SSD focuses on the abnormal behaviors/thoughts/feelings in response to the distressing somatic symptoms The focus is NOT on whether there is a medical explanation for the somatic symptom(s). The person COULD have a medical explanation for the symptom and still have SSD. Differentials: - Body Dysmorphic Disorder BDD - O-C) – but in BDD, it's about appearance not somatic symptom or health - Delusional Disorder (DD somatic type) – but in DD, belief is held with delusional intensity (100% certainty) Different from SSD because there is NO distressing physical complaint, but the patient nonetheless worries about one's health. In SSD, the patient DOES have a distressing physical complaint with excessiveness in response to that distressing physical complaint. Differentials: - Body Dysmorphic Disorder BDD - O-C) – but in BDD, it's about appearance not somatic symptom or health - Delusional Disorder (DD somatic type) – but in DD, belief is held with delusional intensity (100% certainty)

		• Patient performs excessive health-related behaviors or shows maladaptive avoidance			
	Conversion Disorder (“functional neurological syndrome”)	• Altered voluntary motor or sensory function • Evidence of incompatibility between the symptom and neurological conditions • Ie. Psychogenic seizures – pt isn’t pretending because it’s being regulated more subconsciously			Subtypes: • With weakness or paralysis • With abnormal movement • With swallowing symptoms • With speech symptom • With attacks or seizures • With anesthesia or sensory loss • With special sensory symptom (visual) • With mixed symptom
	Factitious	• A person fakes/induces (feigns) physical or psychological symptoms, in self or others, in the absence of obvious “external” rewards. • Satisfaction is from gaining medical attention and being in the sick role (“primary gain”)			Subtypes: • Factitious disorder imposed on Self (Munchausen’s Syndrome) • Factitious Disorder Imposed on Another (Munchausen’s Syndrome, by proxy) Differential: Malingering – person fakes/induces physical or psychological symptoms in self/others for “external” rewards. In this case, complaints cease after gaining reward. Malingering by proxy also possible. Malingering is driven by “secondary gain”.
Trauma and Stressor-related Disorders These disorders share in common psychological distress in response to a stressful event. Traumatic Stressor:	Post-Traumatic Stress Disorder (PTSD)	• Exposure to a traumatic stressor: actual or threatened death or serious injury or sexual violence • Exposure may include direct experience of event, witnessing event, or learning about event to close family member/friend • <i>Greater than 1 symptom from each of the 4 categories of symptoms MUST develop</i> • Intrusion symptoms: 1. Dreams	>1 mo	Psychotherapy: • Supportive therapy offering safe environment to express feelings • Behavioral therapy to address specific problematic behaviors (ie. Avoidance behavior) Medications: (if needed) to reduce emotionality • Antidepressants • Benzodiazepines	Onset: Symptoms usually begin within 3 months of the trauma but can begin ANYTIME Vulnerable populations: PTSD often occurs in young adults but can happen to anyone; Patients with sudden-onset, life-threatening medical events (ie. Heart attack) can experience PTSD

Involves actual/threatened death or serious injury. Non-Traumatic Stressor: Involves situations in which a person perceives that environmental demands exceed one's resources to cope <i>Psychological distress can manifest in many forms (anxiety, dysphoria, anger, dissociation)</i>		- Recollections - Feeling event reoccur - Psychological or physiological distress when encounters symbols Avoidance Symptoms: Symptoms such as avoidance of thoughts, places, or conversations, that are reminders of event Negative alterations in cognition and mood: such as <i>persistent</i> - Negative beliefs/expectations - Negative emotional states - Inability to experience positive emotion - Diminished interest/participation in activities - Detachment/estrangement from others - Dissociative amnesia (common in PTSD but can have other diagnoses when they stand alone) Alterations in arousal and reactivity - Sleep disturbance - Irritable and angry outbursts - Reckless or self-destructive behavior - Concentration problems - Hypervigilance - Exaggerated startle response			Neuroanatomical Correlates: Hyperresponsive amygdala (<i>fear</i>) Underresponsive prefrontal cortex (<i>failure to suppress fear</i>) Reduced volume and dysfunction of hippocampus (<i>failure to suppress fear when in safe contexts</i>) Likely underlies many of the intrusion symptoms and alterations in arousal and reactivity seen in these disorders
	Acute Stress Disorder (ASD) -essentially a short-lived PTSD	- Exposure to a traumatic stressor (actual or threatened death, serious injury or sexual violence) - Numerous PTSD-like symptoms must develop from the four PTSD symptom categories (ie. Intrusive symptoms, avoidance, etc)	3 days – 1 month s/p trauma exposure	Psychotherapy: - Supportive therapy offering safe environment to express feelings - Behavioral therapy to address specific problematic behaviors (ie. Avoidance behavior) Medications: (<i>if needed</i>) to reduce emotionality - Antidepressants - Benzodiazepines	If PTSD symptoms are present, the key to differentiating ASD from PTSD is the time frame: ASD: symptoms start and <i>resolve within the first 30 days</i> after the trauma PTSD: symptoms start any time after the trauma and last <i>more than 30 days</i> Neuroanatomical Correlates: Hyperresponsive amygdala (<i>fear</i>) Underresponsive prefrontal cortex (<i>failure to suppress fear</i>)

					Reduced volume and dysfunction of hippocampus (failure to suppress fear when in safe contexts) Likely underlies many of the intrusion symptoms and alterations in arousal and reactivity seen in these disorders
	Adjustment Disorder	- Development of significant and disproportional emotional/behavioral symptoms in direct response to an identifiable psychosocial stressor (ie. Divorce) – the stressor is often an ordinary life experience but can be traumatic - Causation is inferred by acute onset (symptoms develop within a few months of stressor onset) and brief duration (<i>symptoms are expected to resolve within a few months after stressor termination, if stressor termination is possible</i>) - Symptoms aren't explained by another disorder		- Learning coping strategies in psychotherapy sessions - Involvement in support groups Treatment MAY involve crisis intervention - Hospitalization - Psychotropic medications	Subtypes: - With depressed mood - With anxiety - With disturbance of conduct (externalizing misbehavior such as vandalizing) ALSO: - With mixed anxiety <i>and</i> depressed mood - With mixed disturbance of emotion and conduct - Unspecified (other maladaptive responses) If sufficient symptoms exist after a stressor such that another disorder's diagnostic criteria are met (ie. Major depressive disorder), then that other disorder is diagnosed, NOT adjustment disorder. <i>Adjustment disorder is diagnosed only if there is a casual stressor AND no other disorder explains the symptoms.</i>
	Reactive Attachment Disorder	Inhibited and emotionally-withdrawn behavior towards adult caregivers			
	Disinhibited Social Engagement Disorder (DSED)	Overly familiar behavior with relative strangers (indiscriminate social behavior)			
Dissociative Disorders: Dissociation:	Dissociative Amnesia	Memory loss for autobiographical information, which doesn't occur due to another disorder			Specify if amnesia is "<i>with dissociative fugue</i>" Fugue: Purposeful travel or bewildered wandering associated with amnesia for identity or other

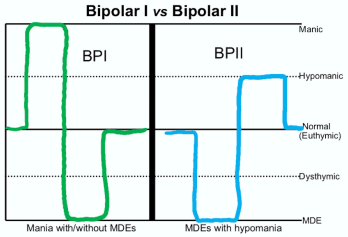
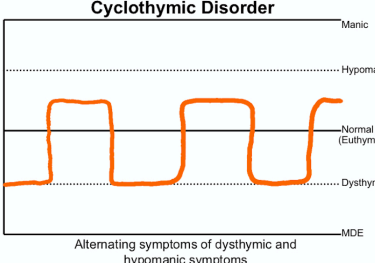
Splitting off from conscious awareness an aspect of self (ie. A memory) usually as a coping strategy for stress		- Memory loss can be: Localized: total loss of personal memory during a circumscribed period  Selective: some (but limited) recall of personal memories during a circumscribed period of time  Generalized: Loss of personal memory of entire life up to and including event 			autobiographical information. Typical features of fugue: - Brief (hours to days) - Unobtrusive lifestyle during fugue - Spontaneous termination of amnesia - Rarely recurs Differential: <i>Organically-based amnesia</i> During a mental status exam (MSE), identify the type of memory problem exhibited - If organic basis: patient will have difficulty learning new information (anterograde memory loss) in addition to past memory loss - If psychological (dissociative) basis: patient learns new information well; only past memory loss (retrograde memory loss) will be present
	Dissociative Identity Disorder (formerly, multiple personality disorder)	- Disruption of identity characterized by greater than or equal to 2 distinct personality states: The primary (host) – not aware of alter(s) - An alter - Inability to recall personal information (as evidenced by frequent memory gaps in host while an alter takes control)			DID Hints: Amnesia with “mistaken identity” experiences and unexpected changes in personal possessions
	Depersonalization/ Derealization Disorder	Either (or both) of the following: Depersonalization: Experiences of unreality, detachment or being an outside observer with respect to one’s thoughts, feelings, sensations, body, or actions Derealization: Experiences of unreality or detachment		Typically involves some form of psychotherapy (ie. Cognitive-behavioral therapy) Hypnosis may be used (cautiously, due to “<i>false memory syndrome</i>”) to help recover memories in a dissociative amnesia	Biological Correlates of dissociation: - Largely unknown - Amnesia: - No structural brain damage accounting for memory loss - Primarily a dysfunction of the process of retrieving memories

		with respect to surroundings (ie. Objects seem unreal or dreamlike) - Reality testing remains intact (ie. Person knows the perceptual experience is just a misperception) - Symptoms: - Result in functional impairment - Are not due to physiological effects of a substance or another medical condition (ie. Seizures) - Are not better explained by another mental disorder, such as panic disorder, acute stress disorder, PTSD, or another dissociative disorder			
Depressive Disorders Share in common <i>low</i> mood without abnormally high mood periods.	Major Depressive Disorder (MDD)	- A person must experience more than 1 major depressive episode (MDE) for a diagnosis of MDD - The person cannot have a history of mania or hypomania (in other words – the person never swings to the opposite “pole” of mood. If the person does swing, then the diagnosis is not MDD. - MDD is known as a ‘unipolar depression’ Major Depressive Episode: *** At least 2 weeks of greater than equal to 5 of the following, one of which MUST be (1) or (2): Depressed Mood	At least 2 weeks	Psychotherapy: a variety of psychotherapies (especially cognitive-behavioral therapy) may help to address some depressive symptoms, especially in milder depressions Antidepressant Drugs (ADs): increase the monoamines (especially serotonin and norepinephrine) to varying extents. <i>Often take 4-6 weeks before observable therapeutic effect</i> <u>1st Line Drugs</u> SSRIs (selective serotonin reuptake inhibitors) – most popular class of ADs with relatively benign side effects (ie. GI disturbance, sexual dysfunction).	Major Depressive Episode symptoms *** can be clustered into three domains: Affective Neuro-vegetative (physical) Cognitive MDD Neurobiology: Genetic Vulnerability – risk of developing MDD - General population: 10% 1st Degree Relative: 20% - MZ co-twin with MDD: 30% - Common Findings Amygdala – ↑ activity HPA axis – ↑ activity (chronic) Cortisol – ↑ levels Cytokines – ↑ inflammatory response

	- Anhedonia (losing interest in previously enjoyable activities) - Significant weight change (usually lose) - Insomnia/hypersomnia (usually insomnia) - Loss of energy - Psychomotor changes - Feeling guilty/worthless - Decreased concentration - Thoughts of death/suicidal ideation Mnemonic for MDE "SIG-E-CAPS" Sleep Change Interest Loss Guilt Energy Problem Concentration Poor Appetite Change Psychomotor Changes Suicidal Ideation		Predominantly block reuptake of 5-HT. (fluoxetine, citalopram, sertraline) SNRIs (serotonin norepinephrine reuptake inhibitors) – similar side effect profile as SSRIs. Not specific to serotonin, hence name. (venlafaxine, duloxetine) 2nd Line Drugs TCAs (tricyclic antidepressants) – prevent presynaptic reuptake of NE and 5-HT. Less serotonin agonism relative to the SSRIs and SNRIs. Has Cardiac side effects (postural hypotension, tachycardia). (imipramine, amitriptyline) MAOIs (monoamine oxidase inhibitors) – prevent enzyme degradation of the monoamines (not selective to serotonin). Diet restrictions to avoid tyramine – induced hypertension. Less widely used. (phenelzine, selegiline) MDD Other treatments: Phototherapy – exposure to bright artificial light – used for season pattern of MDD (seasonal affective disorder) Brain stimulation therapies (ECT, rTMS, VNS)	(relates to sickness behavior) Hippocampus – ↓ volume (due to hypercortisolemia?) Prefrontal cortex – ↓ activity/volume Monoamines – ↓ neurotransmission (treatment focuses on correcting this) In depression, the HPA axis is not being suppressed by a negative feedback loop by cortisol and is therefore activated chronically. This may be caused by a damage to the hippocampus (due to cortisol toxicity) or a pro-inflammatory response to increased cytokines. Cytokines are also known to activate enzymes that destroy serotonin. The prefrontal cortex is unable to inhibit overactivity of limbic regions, adding to the pathophysiology. Specifiers for Mood Disorders - With Melancholic features - With Atypical Features (sleeping more, eating more) - With Psychotic Features - Mood Congruent - Mood Incongruent - With catatonia - With peripartum onset - With seasonal pattern - With Rapid Cycling
Persistent Depressive Disorder	- Chronic depressed mood for greater than 2 years The depressed mood can be in the form of: - A long-lasting MDE OR Dysthymia: Chronic depressed mood plus greater than 2 of the following symptoms - Poor appetite or overeating	Greater than 2 years	Same psychotherapeutic and pharmacological methods used for MDD are used for the dysthymic subtype of PDD. Brain stimulation therapies used for depression may also be used for PDDs that involve treatment-resistant and/or severe MDEs.	Specifiers to describe the <i>nature</i> of pt's depression during the 2-year interval PDD with persistent MDE: depression is characterized by a prolonged MDE PDD with pure dysthymic syndrome: depression is

		- Insomnia/hypersomnia - Low energy - Low self-esteem - Trouble concentrating or making decisions - Feelings of hopelessness			characterized by prolonged dysthymia but no MDE occurs PDD with intermittent MDEs: depression is characterized by dysthymic periods and MDEs Subtypes of PDD
	Premenstrual Dysphoric Disorder	Symptoms must - Present in the week before menses onset - Improve a few days after menses onset - Minimize in the week post-menses Symptoms include - Mood lability, irritability, dysphoria, and anxiety symptoms - Anhedonia, problems concentrating, lethargy, appetite and sleep change, physical symptoms (breast tenderness, weight gain)	Week before menses	SSRIs	Symptoms must cause clinically significant distress or functional impairment (ie. Symptoms beyond “pre-menstrual syndrome”). -Validated clinical scales are used to confirm the diagnosis
	Disruptive Mood Dysregulation Disorder	Core Features - Sever temper outbursts at least 3 times/week - Sad, irritable or angry mood almost daily - Reaction is disproportionate to situation	- Child must be at least 6 years old - Symptoms must begin		New to DSMV5 <i>Many questions as to whether the diagnosis is a <u>valid entity</u></i> DMDD diagnosis was developed so that a child’s irritability would not be mislabeled as a part of a bipolar disorder

		Symptoms are present in multiple settings	BEFORE age 10 NOT diagnosed after age 18		- Labeling DMDD kids with a depressive disorder (when not depressed) may be just as inaccurate and lead to inappropriate treatment - Many arguments that this should fall under a behavioral disorder category
Bipolar and Related Disorders Disorders in which high mood is the common theme. Manic Hypomanic Euthymic Dysthymic Depressed Low mood may also be required	Bipolar I disorder ("manic-depression") - A person must experience at least 1 manic episode Manic episode = abnormally elevated mood (or irritability) PLUS increased energy At least 3 additional symptoms are required 1. Inflated self-esteem/grandiosity 2. Decreased need for sleep 3. Pressure speech (nonstop) 4. Flight of ideas (and/or racing thoughts) 5. Distractibility 6. Increased goal-directed activity or psychomotor agitation (purposeless activity) 7. Excessive involvement in "risky" activities Mnemonic for Manic Symptoms: DIG FAST Distractibility Irresponsibility Grandiosity Flight of ideas A – increased <u>a</u>ctivity S – decreased need for <u>s</u>leep Talkativeness	At least 1 week (or any duration if hospitalization is required)	Pharmacological Treatment Mood stabilizers Lithium: side effects are hand tremors, polydipsia, polyuria Anticonvulsants (valproic acid) Antipsychotics (quetiapine) May be used alone or in combination. Drugs have variable mechanisms of action Antidepressants are not FDA-approved for BP depression. They are contraindicated due to the risk of mania induction. If used off-label, then a mood stabilizer is added to help prevent the mood swing	Symptoms cause marked impairment in functioning Most BPI patients <u>also</u> experience MDEs but MDEs are not a necessary for a BPI diagnosis (despite the term "bipolar") BPI Possibilities Manic Episodes without MDEs (uncommon) Manic Episodes with alternating MDEs (common) Etiology: - Genetic vulnerability General Population: 1% 1st degree relative: 20% Monozygotic co-twin: 80% BPI is more genetically controlled than MDD. - Manic episodes involve increased monoaminergic activity	
	Bipolar II disorder A person experiences at least 1 MDE and at least 1 hypomanic episode	Hypomania must last greater than	Mood stabilizers Use those drugs that are particularly effective in treating	BPII must have MDEs <i>with</i> hypomania	

		Hypomanic episode – hypomania includes the same symptoms of mania but of different <i>severity</i>. Mania causes ‘marked’ impairment in functioning, but hypomania does not. Mania can involve psychosis, but hypomania does not. Change in mood must be <i>unequivocal and uncharacteristic</i> of person.	or equal to 4 days.	the depressed phase of BP II (more problematic phase than hypomania). Quetiapine – an antipsychotic drug that is FDA-approved for bipolar depression. Taken alone or with lithium or a mood-stabilizing anticonvulsant (i.e. Valproic acid)	 The chart compares Bipolar I (BPI) and Bipolar II (BP II) mood states. BPI shows a green line with a high peak for Mania and a low trough for MDE. BP II shows a blue line with a lower peak for Hypomania and a low trough for MDE. The Normal (Euthymic) state is represented by a horizontal line. The chart is divided into two sections: 'Mania with/without MDEs' for BPI and 'MDEs with hypomania' for BP II.
	Cyclothymic Disorder	- For greater than 2 years, a person experiences periods of hypomanic symptoms that fluctuate with periods of depressive symptoms. - Criteria for an MDE or a manic episode have never been met. - Cyclothymia is similar to BPI, but the mood shifts are not as extreme	Greater than or equal to 2 years	Mood stabilizers	 The chart shows the mood fluctuations in Cyclothymic Disorder. An orange line oscillates between levels of Hypomania and Dysthymia, never reaching the levels of Mania or MDE. The Normal (Euthymic) state is shown as a horizontal line. The x-axis is labeled 'Alternating symptoms of dysthymic and hypomanic symptoms'.
Sleep-Wake Disorders	Insomnia Disorder	Difficulty initiating or maintain sleep	More than or equal to 3 mos	Improve sleep hygiene: Behavioral strategies such as: - Lie down only when tired - If not asleep in 10 minutes, depart bed - If early awakening, don't stay in bed - Maintain consistent sleep schedule - Avoid caffeine, esp after noon - Exercise more than 2 hours before bedtime - Take hot bath within 2 hours of bed time Pharmacological Approaches Sedatives (benzodiazepines like diazepam) – induces sleep and increases sleep duration. This treatment option is recommended	Etiology: Insomnia is a learned arousal response (classical conditioning) to bedtime and other cues associated with the sleep environment. The learned arousal results from inappropriate sleep hygiene (ie. Using bed for wakeful activities)

				only for short-term use (2-4 weeks) due to long-term side effects such as: ○ Poor sleep quality due to decreased slow wave sleep (N3) and REM ○ Tolerance and withdrawal ● Benzodiazepine-like drugs (ie. Zolpidem) – which usually have fewer side effects	
Hypersomnolence Disorder	● Excessive sleepiness despite sufficient sleep (at least 7 hours) ● Average sleep episode = 9.5 hours ● Unrefreshing naps ● Normal PSG	More than or equal to 3 mos	Stimulants ● Methylphenidate Stimulant-like drugs ● Modafinil	Neuropathology ● Unknown etiology (idiopathic form) ● Exclude other causes (ie. Narcolepsy)	
Narcolepsy	● Recurrent irresistible sleep occurring within the same day, several times per week, for more than or equal to 3 mos AND ● At least 1 of the following: ○ Cataplexy: sudden loss of muscle tone while awake precipitated by emotion. Considered an aberrant manifestation of REM sleep ○ Hypocretin (orexin) deficiency: spinal tap needed. Orexin is a hypothalamic neuropeptide. Deficiency may be autoimmune related, where orexin producing cells are destroyed. ○ Characteristic PSG abnormalities (either of the following:	More than or equal to 3 mos	Polytherapy 1. Stimulants (ie. Dextroamphetamine) or stimulant-like drugs to promote wakefulness (ie. Modafinil) 2. Antidepressants for cataplexy (SSRIs – citalopram) Monotherapy: Xyrem (sodium oxybate) – controversial (GHB = gamma hydroxybutyrate) ● GHB (schedule I drug) except when marketed as Xyrem and used for narcolepsy ● Xyrem (schedule II drug) treats cataplexy and somnolence ● Available via a restricted distribution service (expensive and needs to be delivered to pt via courier)	Note: other REM-related behaviors may be present but are NOT diagnostic criteria: ● Sleep paralysis ● Hypnagogic (upon falling asleep) hallucinations ● Hypnopompic (upon awakening) hallucinations	

		1. Nocturnal PSG : short REM-seep latency (sleep-onset REM) – pt goes into REM sleep in less than 15 minutes 2. Daytime PSG – Multiple sleep latency test – MSLT – short REM-sleep latency AND short sleep-onset latency			
	Breathing-Related Sleep Disorder 1. Obstructive sleep apnea/hypopnea (OSA)	• Multiple episodes of breathing cessation/reduction (of around 10 seconds) occur per night due to an upper airway obstruction ○ Obstruction usually occurs when the soft tissue in the back of throat collapses during sleep		Continuous Positive Airway Pressure (CPAP) • A device that maintains an open airway by delivering compressed air at a specific air pressure to the mask's nasal pillow Additional approaches to maintaining open airway at night include • weight loss • avoiding back sleeping • orthodontic devices • surgery	Apneas/hypopneas cause sleepiness • The rise in CO₂ during apneas causes temporary arousal (not awakening) from sleep which bumps the person from a deep to a light stage of sleep (ie. from 4-2). • Duration of sleep may be adequate but the sleep is unrefreshing Classic Profile: Middle-aged, overweight male who snores loudly and intermittently
	2. Central Sleep apnea (CSA)	• Multiple episodes of cessation of breathing per night caused by CNS dysregulation of breathing		Varies depending on the cause (ie. Respiratory stimulants such as acetazolamide , or nocturnal oxygen)	Multiple causes: • Primary – <i>idiopathic</i> • Opioid use A PSG distinguishes OSA from CSA based on whether thoracic movements occur at the start of an apneic episode: • OSAH – thoracic effort occurs • CSA – no thoracic effort occurs
	Circadian Rhythm Sleep-Wake Disorder	• Excessive Sleepiness or insomnia resulting from a mismatch between a person's circadian		Phototherapy at strategic times during the day to adjust the timing of the sleep-wake cycle	

		sleep=wake pattern and the sleep-wake schedule required by the environment • Issue = timing of circadian rhythm, not necessarily that the individual is tired • Example: delayed sleep onset and awakening times, with the inability to fall asleep and awaken at a desired earlier time		• Setting of Circadian Clock (governed by SCN) • Light -> SCN -> inhibits pineal gland -> decreases melatonin -> alert • No light -> SCN -> activates pineal gland -> increases melatonin -> drowsy	
	Parasomnias: 1. Non-Rem Sleep Arousal Disorder (N-RSAD)	Repeated episodes of incomplete awakening from sleep with either of the following: • Sleep walking (somnambulism): rising from bed and walking about with a blank and staring face, relative unresponsiveness, and difficulty awakening • Sleep terrors: Abrupt terror arousals (usually with panicky scream), intense fear and autonomic arousal, and unresponsiveness to comforting by others – usually can't remember because they are often awoken from deep sleep Occurs in 1st 1/3 of sleep		Benzodiazepines (if needed) to decrease SWS If a sleep walker, then consider environmental safety	• Episodes Occur in the first 1/3 of sleep (during slow wave sleep [SWS]) • Little to no dream imagery • Amnesia for the episodes
	2. Nightmare Disorder	• Extremely dysphoric dreams that typically involve threats to survival, security, or physical integrity • Occurs in the 2nd half of sleep in REM		Antidepressants to decrease REM	• Rapid alertness upon waking • Dream content is well remembered • Good recall of the awakening the next morning
	3. REM Sleep Behavior Disorder (RSBD)	• Vocalizations and/or complex motor movements occur during REM sleep • REM sleep without atonia is confirmed by PSG		• Clonazepam (a benzodiazepine): the therapeutic mechanism of action is unclear • Modification of sleep environment for safety	RSBD Features: • Typically action-filled, violent dreams • Immediately awake, oriented and alert with detailed dream recall

		The disturbance is not induced by a substance			Loss of motor inhibition during REM is associated with neurodegenerative disease (ie. Parkinson's disease, Lewy body dementia)
	4. Restless Legs Syndrome (RLS)	Urge to move legs in response to uncomfortable sensations with all of the following features: - Occurs/worsens during inactivity - Nocturnal worsening of symptoms - Temporary relief from discomfort by moving The patient is aware of symptoms and complains of insomnia		Parkinson's disease drugs (also benzos, anticonvulsants , etc)	This is a volitional leg movement in response to tingling feeling while awake. Difficulty falling asleep leads to EDS
	5. Periodic Limb Movements (PLMs)	Repetitive muscle contractions during sleep, usually of the lower limb - Associated with, multiple sleep stage arousals - Patient complains of daytime sleepiness but is unaware of movements - Electromyogram during PSG confirms diagnosis - Treat using similar drugs as for RLS			Note: Sleep starts (a.k.a. hypnic or myoclonic jerks) are muscle contractions that occur only upon falling asleep; these are normal and not diagnosable
Eating Disorders Screening for Eating Disorders "SCOFF" Questions Sick (induce vomiting?)	Anorexia Nervosa	Restriction of food that leads to being significantly underweight (based on BMI) 18.5 BMI is the lower limit of normal; however, use clinical judgment with BMI cut offs: Adults:		A patient is typically not self-referred, since anorexia tends not to be distressful. In-Patient Hospitalization: Behavioral technique (reinforcement/punishment) are utilized to achieve the immediate goal of weight gain	Body dysmorphic disorder is an inherent criterion for anorexia Subtypes: based on predominant current behavior (not longitudinally/historically) regarding binging/purging:

• Control (lose control?) • One (1 “stone” weight loss in 3 mos) about 14lbs/6.5kg • Fat (believe fat?) • Food (food dominates?) <i>Yes to greater than or equal to 2 items of SCOFF suggests an eating problem</i>		○ BMI <17 (clearly below normal) ○ BMI greater than 17 but around 18.5 (gray zone) Children (2-18yrs): ○ BMI <5th percentile • Intense fear of weight gain (evidenced by extreme preoccupation and frequent assessment, behavior that interfere with weight gain, etc) • Body image disturbance (ie. Belief of being overweight, or denies the seriousness of low weight)		• At attendant often supervises the patient to thwart attempts to lose weight Psychotherapy (in and out patient) : cognitive-behavioral therapy – the goal is to change patient’s attitude about food/weight and stop their restrictive eating habits Pharmacological: limited treatment of medical complications and comorbid psychiatric conditions Outcome: • Good short-term response to treatment (ie. Weight gain) in a controlled, in patient setting. • Long term prognosis tends to be poor – patients tend to relapse once released into their home environment	• Binge-eating/purging type: The person recurrently binges and/or purges • Restricting type: The person does not recurrently binge or purge* (weight loss is primarily through dieting, fasting, and or/exercise) *Purge Definition: Use of vomiting, laxatives, diuretics, or enemas or eliminate calories Associated Features: • > 90% are women; 1-2% of women are affected; women in occupations that encourage thinness are at greater risk (modeling) • Symptoms usually begin in adolescence and have a chronic course • Many minor and major medical complications can result (hormone deficiency, organ failure, CNS changes) with a high mortality Indicators of excessive thinness • Low BMI • Amenorrhea and loss of sex drive • Constipation • Hypothermia and lanugo (fuzzy + fine hair because) • Bradycardia and hypotension • Hypercholesterolemia – due to internal metabolism of cholesterol being slowed down • Anemia • Leukopenia • Low bone mineral density (osteoporosis) Indicators of excessive vomiting: • Calloused knuckles (Russell’s sign)
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					- Dental enamel erosion - Salivary gland inflammation (chipmunk cheeks) - Subconjunctival hemorrhage - Hypokalemia Note: Vomiting commonly occurs in Anorexia and Bulimia, but it isn't an essential symptom of either disorder Etiology of Anorexia <i>Psychological and Social Factors</i> - Dieting due to cultural pressures to be thin - Using dieting as a means to "control" one's life (especially when establishing autonomy from parents in adolescence) - A perfectionist personality trait leading to obsessive dieting - Weight loss is reinforced (internally and externally) <i>Biological Factors</i> - There is a genetic susceptibility to anorexia - A person may inherit biochemical alterations leading to obsessiveness - Cultural pressures may direct the obsessiveness to body image
	Bulimia Nervosa	- Recurrent binge eating - Recurrent inappropriate compensatory behavior for binge - Purging (vomiting, laxatives, diuretics, enemas) - Non-purging (fasting and/or excessive exercise) - The binge/inappropriate compensatory behaviors must	> or equal to 1/wk for 3 mos	<i>A patient typically initiates treatment, since the disorder distresses the patient</i> - Treatment is usually out-patient based Cognitive-behavioral therapy is essential - Antidepressant medications may be a useful adjunct to help with impulse dyscontrol (SSRIs)	Definition of a binge: - Eating a large amount in a discrete period (ie. An amount "definitely larger than most people would eat under similar circumstances") - Eating is out of control during episode Features: - There are no subtypes for Bulimia - Bulimics typically have normal body weight or are slightly overweight

		occurs > or equal to 1/wk for 3 mos Self-evaluation is unduly influenced by body shape and weight		With greater motivation to change, the prognosis is better for Bulimia than Anorexia	- More than 90% of Bulimics are women with 2-3% of women affected Course - Symptoms tend to begin in adolescence and be chronic - Women often have a history of both Anorexia and Bulimia, but at different times - Medical complications tend to be fewer and less severe than for Anorexia, but morbidity and mortality may still result Etiology of Bulimia <i>Psychological and Social Factors</i> - Dieting may result from societal pressure to be thin and/or as a means to improve self-esteem - Strict dieting results in a broken diet and impulsive “all or none” eating (ie. Binging) - Fear of weight gain after binge leads to inappropriate compensatory behavior <i>Biological Factors</i> - A person may be inheriting biochemical changes that leads to impulse dyscontrol - Low (unstable) serotonin is associated with this disorder
	Binge-Eating Disorder (BED)	- Binge Eating (as per bulimia criteria) at least 1/wk for 3 mos - Binging is associated with behaviors such as - Rapid eating - Eating until uncomfortably full - Eating when not physically hungry		Cognitive-behavioral therapy Antidepressant drugs Stimulant drug lisdexamfetamine (Vyvanse)	Features of BED - BED is often associated with obesity (BMI > or equal to 30) but obesity isn't a diagnostic requirement. - BED is a distinct problem from obesity - Not all obese people binge eat - There is greater distress and functional impairment in BED than in obesity

		○ Eating alone due to shame about quantity ○ Feeling disgusted/ guilt/ depressed after binge • No inappropriate compensatory behavior (such as purging)			• Health consequences of BED are similar to those of obesity Epidemiology: • ~2% of population; less skewed female gender bias Typical Onset & Course: • Adolescence with chronic course • Cross-over to other eating disorders is uncommon Etiology • Unknown but different from Bulimia (in BED, <i>dysfunctional dieting doesn't precede binging</i>)
	Unspecified Eating Disorder	• Subclinical eating problems emulating any of the aforementioned eating disorder criteria but not enough to make a diagnosis			Examples: • A woman with a BMI=22kg /m² frequently purges but does not binge eat • A woman with a BMI = 25kg/m² binges and purges but does so only once per month
Gender Disphoria	Gender Dysphoria (GD) -transgender, transsexual	• A marked incongruence between one's assigned (natal) gender and one's experienced gender • The incongruence results in clinically significant distress or functional impairment • Indicators of GD: ○ Insistence that one is the experienced gender ○ Desire to act as, and be treated as, the experienced gender ○ Desire to be rid of one's sex characteristics and to possess those of the experienced gender	At least 6 months	Management of chronic GD cases involves facilitation of: • Transgender living • Hormone treatment • Sex reassignment surgery (if eligible): ○ Phalloplasty (creation of penis) ○ Vaginoplasty (creation of vagina) ○ "gender conformation surgery" – more PC term used	

		○ Conviction that one has the typical feelings/reactions of the experienced gender			
Sexual Dysfunctions	Female sexual interest/Arousal Disorder	• Problems with libido and/or physiological arousal		To increase libido and responsiveness: • Dual Sex Therapy: involves sensate focus exercises (prescribed non-sex activities progressing to coitus) To slow ejaculation in premature ejaculation: • Squeeze technique: partner squeezes the glands penis to cause discomfort • Start and stop technique: partner ceases stimulation of the partner to reduce pleasure • SSRIs PLISSIT: Overall treatment strategy • Permission: ask about sex life (this gives patient approval to discuss, fix, and enjoy sex) • Limited information: provide basic sex education and correct misconceptions • Specific Suggestions: Provide specific suggestions (sensate focus exercises) • Intensive Therapy: Refer on if the above steps fail to resolve the issue	Specifiers for Sexual Dysfunctions: Indicate whether dysfunction is: • Generalized or situational • Life-long or acquired Causes of Sexual Dysfunction • Psychological factors are primary (i.e. Anxiety, guilt, body image) • A medical condition may co-exist but insufficiently explains the symptom Assessment of Sexual Dysfunction 1. Taking a sexual history: doctors should routinely ask questions about gender and sexual functioning in a neutral, non-leading, non-judgmental fashion Terminology: • Gender identity: (cis gender, transgender) refers to one's psychological sense of being male/female • Sexual preference/sexual orientation (homosexual, heterosexual, bisexual) refers to whom the person is sexually attracted 2. If sexual dysfunction is acknowledged, consider whether the symptoms are related to: • A medication (ie. SSRIs) • Alcohol/drug use (sedatives)
	Female Orgasmic Disorder	• A delay in, the inability to, or low-intensity orgasms			
	Genito-Pelvic Pain/Penetration Disorder	• Difficulties with vaginal penetration or vulvovaginal/pelvic pain during intercourse/penetration attempts			
	Male Hypoactive Sexual Desire Disorder	• Problems with low libido			
	Erectile Disorder	• Problems maintaining or attaining an erection			
	Premature Ejaculation	• Ejaculation occurring during partnered activities within about 1 minute following vaginal penetration			
	Delayed Ejaculation	• Marked delay in, or absence of, ejaculation • Can be caused by taking SSRIs			

					• A medical/physical problem (circulatory problems leading to erectile dysfunction) • Normal aging (slower erections, decreased orgasm intensity, increased refractory period) 3. Be familiar with tests to differentiate a psychiatric vs. organic basis for sexual problems <i>Example: erectile disorder – the Snap Gauge Test: determines whether erections occur during REM. If REM erections occur, then problem is not organic</i>
Paraphilic Disorders Paraphilia: an intense and persistent (> or equal to 6 mos) deviant sexual interest (as manifested by fantasies, urges, or behaviors) Deviant as per DSM-5 – sexual interest other than for genital stimulation or preparatory fondling with phenotypically normal, physically	Exhibitionistic Disorder	• Sexual arousal from exposing genitals to an unsuspecting stranger		Behavioral Therapy • Aversion Therapy: While engaging in. (or thinking about) one's paraphilia, an ACTUAL noxious stimulus is applied to the patient OR While fantasizing about one's paraphilia, a patient IMAGINES an aversive consequence (<i>covert sensitization</i>) • Medication: Antidepressants: SSRIs are used to reduce libido, control impulses and reduce obsessions Anti-androgens: "chemical castration" – medroxyprogesterone acetate "depo-provera" is used to decrease testosterone levels -helps reduce sex drive and thus minimize deviance	Paraphilic "disorder" A paraphilia is a disorder if <i>any</i> of the following three conditions occur: - Acts on the urge with a non-consenting person - Acts on the urge with a consenting person (or acts on an urge unrelated to people) and the actions cause significant distress/functional impairment - Does NOT act on the urge but the urge causes significant distress/functional impairment Etiology of Paraphilias The cause of sexual deviancy is multi-factorial and largely unknown. Suggested influences:
	Voyeuristic Disorder	• Sexual arousal from observing unsuspecting person either naked, In the process of disrobing, or engaging in sexual activity • Perpetrator must be at least 18 years old			
	Fetishistic Disorder	• Sexual arousal from nonliving objects or a highly-specific sexual focus on a non-genital body part <i>Note: Devices specifically designed for tactile genital stimulation don't count as a fetish</i>			
	Transvestic Disorder	• Sexual arousal by cross-dressing (wearing the opposite sex's clothing) • This is not about gender identity			

mature consenting human partners	Frotteuristic Disorder	Sexual arousal from touching or rubbing against a non-consenting person		-used especially when sexual behavior is illegal, uncontrolled, or dangerous -drug non-compliance is a problem	- Testosterone levels affecting sex drive (no differences have been established) - Frontal lobe dysfunction and serotonin dysregulation affecting impulse control (obsessive-compulsive ish) - Classical conditioning (i.e. The object/situation was previously paired with sexual excitement and now triggers the same sexual excitement)
	Sexual Sadism Disorder	Sexual arousal from causing psychological/physical suffering of another person			
	Sexual masochism disorder	Sexual arousal from receiving psychological/physical suffering			
	Pedophilic Disorder	Sexual Arousal by a prepubescent child (usually 13 years of younger) <i>Note: The perpetrator must be at least 16 years old AND at least 5 years older than the child</i>			
Schizophrenia Spectrum and Other Psychotic Disorders Psychotic: Implies a break from reality in thinking and/or perceptions ("thought" disorders) Each diagnosis involves one or more of the following 5 psychotic symptom domains: 1. Delusions 2. Hallucinations	Schizophrenia	An active-phase for greater than or equal to 1 month, defined as: - At least 2 psychotic domain symptoms with at least one being a core symptom (1-3) Delusions Hallucinations (visual, auditory, command) Disorganized speech - Grossly disorganized behavior - Negative symptoms At least 1 symptom persists for greater than equal to 6 months <i>(doesn't have to have 2 the whole time)</i>	Active phase for greater than or equal to 1 month, at least 1 symptom persists for greater than equal to 6 months	<i>*Look at one pager for Schizophrenia for details</i> Antipsychotics: D2 receptor antagonist (traditional) - Chlorpromazine (thorazine) - Haloperidol (Haldol) Antagonists of D2,3,4 and 5-HTA_{2A} receptors (atypical) - Clozaril (Clozapine) – 1-2% agranulocytosis risk - Risperidone (Risperdal) - Olanzapine (Zyprexa) Supportive Interventions - Vocational rehabilitation - Individual psychotherapy - Family education ECT Augmentation: Used for medication-refractory schizophrenia (especially catatonic type) Psychosurgery: No longer performed on schizophrenics	Schizophrenia Timeline (elaborated) Duration: Signs of the disturbance for greater than or equal to 6 months (including any prodromal and/or residual phases): Greater than or equal to 6 months: Prodromal phase (before having an activate phase) Active phase (must be greater than or equal to 1 month) Residual phase Prodromal and residual phases often manifest as only 1 symptom domain (ie. Negative symptoms) Subtypes: DSM no longer subtypes schizophrenia (except to specify "with catatonia") Nonetheless, the presentation of schizophrenia can dramatically vary depending on which psychotic symptoms are most prominent Differential: Substance-induced psychosis (i.e. consider psychotomimetic drugs such as stimulants and dissociative anesthetics) <i>*Look at one pager for Schizophrenia</i>

3. Disorganized Speech 4. Disorganized behavior 5. Negative <i>*Refer to psychotic symptom domain one pager</i>	Schizophreniform Disorder	- At least 1 month of Active-Phase Symptoms - Signs of the disturbance last LESS than 6 months	More than 1 month, less than 6 months		
	Brief Psychotic Disorder	- At least 1 core psychotic domain symptom (delusions, hallucinations, disorganized speech) - Episodes lasts < than 1 month - Person returns to full premorbid functioning - Specify “with marked stressor” or “without marked stressor” based on the trigger of episode	< than 1 month		
	Delusional Disorder	- The presence of a delusion - Symptom must last greater than or equal to 1 month - No other psychotic domain symptoms - No functional impairment aside from the direct impact of the delusion	greater than or equal to 1 month	- Antipsychotics - Trial separation from partner (in case of shared delusions) - Increasing sources of reality testing	Official Subtypes: Persecutory Type: Belief of malevolent treatment Grandiose Type: belief of having some great (but unrecognized) talent or insight or having made some important discovery Erotomantic type: belief of somebody being in love with the individual Jealous Type: belief of infidelity by partner Somatic Type: Belief involving bodily functions/sensations, such as parasitosis, malodorous, organ dysfunction) Differentials: Body Dysmorphic Disorder (BDD) with absent insight (delusional belief): in BDD, delusion is appearance related Illness Anxiety Disorder (IAD): IAD patients worry about health but they are not delusional Unspecified type (theme is none of the above)

					Specify if delusional disorder is “with bizarre content” • Non-bizarre delusion: A belief that involves something that could occur in real life (belief is physically possible) • Bizarre delusion: A belief that is clearly implausible and not derived from ordinary life experiences A delusion may develop in the context of a close relationship with a psychotic individual. The person believes the same delusion as the psychotic individual. • The person is diagnosed with Delusional Disorder (formerly “shared psychotic disorder”)
	Schizoaffective Disorder	• Active Phase of Schizophrenia occurs concurrently with a major mood episode (an MDE or manic episode) • Active phase lasts greater than or equal to 2 weeks without mood symptoms • Mood symptoms are present for “the majority of the total duration” of symptomatic periods of the illness		• Antipsychotic medications • Supportive Psychotherapeutic/psychosocial interventions	Shorthand Approach to Diagnosis: • Concurrent psychosis and mood? • Psychosis alone? • Mood symptom duration proportional with psychotic symptom duration? Subtypes: • Bipolar Type (if mania is experienced) • Depressive Type (if depression is experienced but not mania)
Personality Disorders Pattern of perceiving, relating, and thinking about the environment and oneself that is:	Paranoid Personality Disorder (PPD)	• Suspicion that others are being exploitative or deceptive • Unjustified doubts about loyalty/trustworthiness • Reluctance to confide • Reads hidden threatening meanings into remarks • Reacts angrily to perceived insults		• Cognitive Behavioral psychotherapy is typically used as the primary form of treatment. • Medications could be used as an <i>adjunctive treatment</i> In general, treatment is difficult due to patient disinterest in treatment	Differentials: Delusional Disorder (DD) – Persecutory type • In DD, the paranoid thought is believed with certainty but in PPD, the paranoid thought is merely <i>suspected</i> • In DD, the onset is sudden and typically in middle age with a

- Chronic and pervasive - Culturally deviant - Maladaptive or distressful Onset & Course: In childhood-adolescence with behaviors solidifying with age (typically not diagnosed until adult) Cluster A: (<i>Odd-eccentric</i>) - Paranoid - Schizoid - Schizotypal Cluster B: (<i>dramatic/emotional/erratic</i>) - Histrionic - Borderline - Narcissistic - Antisocial Cluster C: (<i>anxious/fearful</i>) - Avoidant - Dependent - Obsessive - compulsive		Theme: Distrust and Suspiciousness		and the inherent difficulty in unlearning entrenched behaviors.	focus of a single paranoid theme but in PPD, symptom onset is gradual and typically in childhood with <i>pervasive paranoia</i>
	Schizoid Personality Disorder	- Lacks desire for close relationships/intimacy - Lacks close friends - Prefers solitary activities - Derives pleasure in few activities - Indifference to praise/criticism - Emotional coldness, detachment or flat affect Theme: Interpersonal detachment			Not particularly bothered by anything, just don't be around people/have no interest. More distressful for people around them than to actual person
	Schizotypal Personality Disorder	- Ideas of reference - Odd beliefs or magical thinking - Unusual perceptual experiences - Odd thinking and speech (vague, metaphorical) - Suspiciousness - Odd appearance (including affect/behavior) - Problematic social relatedness (lack of close friends, excessive social anxiety) Theme: eccentric			Schizotypal behaviors are on the schizophrenia spectrum: - This disorder is listed in both the "personality Disorders" and "Schizophrenia Spectrum & Other Psychotic Disorders" chapters of DSM-5 - Schizotypals MAY eventually develop psychotic symptoms (thus, schizotypal personality <i>may be the premorbid personality of schizophrenia</i>) but some never develop schizophrenia Schizotypal – usually not as sudden as Schizophrenia onset and no actual delusions/hallucinations
	Histrionic Personality Disorder	- Needs to be the center of attention - Uses physical appearance for attention - Inappropriately provocative behavior - Shallow and rapidly shifting emotions			

		• Impressionistic Speech but lacks detail • Exaggerated emotional expression • Considers relationships intimate (best buddies with everyone but in actuality, not really) Theme: Excessive Emotionality			
Borderline Personality Disorder	<u>Relationships:</u> • Unstable, intense relationships with alternating idealization and devaluation ('splitting') • Frantic avoidance of abandonment <u>Emotions</u> • Intense, uncontrolled anger • Marked reactivity of mood • Chronic feelings of emptiness <u>Self</u> • Unstable self-image <u>Behaviors</u> • Self-damaging impulsivity • Suicidal gestures/self-mutilation • Transient stress-related dissociation/paranoia Theme: Instability		Dialectical Behavioral Therapy (DBT) – <i>specific for borderline personality disorder</i> Emphasis is on self-acceptance yet the need to change. Involves cognitive-behavioral techniques (4 “modules”) • Mindfulness: to be present in the current moment and accept one’s thoughts and emotions) • Distress tolerance: To increase one’s tolerance of negative emotions rather than acting on them • Emotion Regulation: to change/manage problematic emotions • Interpersonal effectiveness: to communicate in an assertive self-respectful way that strengthens relationships	Goal: To help become aware moment-by-moment of the sequence of emotions and thoughts that preceded destructive behavior with the goal of breaking the sequence and regulating emotions without acting on them	
Narcissistic Personality Disorder (NPD)	• Grandiose sense of self-importance • Preoccupied with success • Requires admiration • Sense of entitlement • Interpersonally exploitative				Differentials: 1. Delusional Disorder (DD), Grandiose Type In DD< the person typically has a single grandiose belief of delusional proportion, but in NPD, the person shows <i>general arrogance</i>. DD

		Lacks empathy with haughty attitudes Theme: Grandiosity A “narcissistic injury/blow or threat to self-esteem” may result in a rage reaction			patients also have an abrupt and older age of onset of symptoms than NPD. 2. Bipolar I (BPI) Disorder: BPI involves a mood disturbance with more symptoms than grandiosity (like pressured speech/lack of need for sleep)
	Antisocial Personality Disorder (ASPD)	- Repeated unlawful acts - Deceitfulness (manipulative) - Irritability and aggressiveness - Reckless disregard for safety of self or others - Irresponsibility (employment or financial) - Lack of remorse Person must be at least 18 for ASPD diagnosis and there must be evidence of Conduct Disorder with onset before age 15 Theme: Pervasive disregard for others’ rights			ASPD is associated with lack of physiological reactivity to stressors (? Amygdala dysfunction) – reason why they can often pass lie detector tests. “Psychopath” (sociopath) is often used synonymously with ASPD, but psychopaths are only a research subset of ASPD. Psychopaths are usually the worst of the worst of anti-socials.
	Avoidant Personality Disorder	- Avoids interpersonal and occupational activities for fear of criticism (fear of negative evaluation) - Unwilling to start relationship unless certain of being liked - Restraint within relationship for fear of ridicule - Reluctance to engage in new activities - Negative self-image Theme: inferiority complex			Differentials: 1. Schizoid Personality Disorder (SPD): People with schizoid personality have no desire for a relationship but those with Avoidant Personality Disorder personally desire a relationship, just afraid of negative evaluation. 2. Social Anxiety Disorder (SAD): Anxiety disorder characterized by excessive fear of negative evaluations. SAD pts have higher amounts of sympathetic nervous system arousal. Avoidants have some anxiety but not the extreme levels seen in SAD.

	Dependent Personality Disorder	- Indecisive - Others must take responsibility for life - Difficulty disagreeing - Difficulty initiating due to low confidence - Excessive lengths to keep/gain support - Feels helpless when alone - Urgently seeks another relationship if one ends Theme: Excessive need to be cared for			
	Obsessive-Compulsive Personality Disorder (OCPD)	- Preoccupation with details, rules, lists until point of activity is lost - Perfectionism interferes with task completion - Excessive devotion to work - Rigid and stubborn - Over-conscientious, scrupulous - Reluctance to delegate or to work with others - Hoarding and miserly behavior Theme: inflexible perfectionism & control			Differential Obsessive-Compulsive Disorder (OCD) - OCD (which falls in the O-C & related disorders chapter) involves uncontrolled intrusive thoughts and/or repeated rituals. - OCPD is characterized by excessive perfectionism and control, similar to the “type A” personality (not a DSM term but a research construct) Characteristics: Competitive, time urgent, pressured, impatient, irritable, hostile Association: Coronary heart disease
Disruptive, Impulse-Control and Conduct Disorders (DICCDS) Common features of these disorders:	Oppositional Defiant Disorder (ODD)	Behaviors/emotions characterized by Angry/irritable mood: often loses temper, easily annoyed, resentful Argumentative/Defiant Behavior: often argues/refuses to comply with authority, deliberately annoys, blames Vindictiveness			

- Problems with self-control of emotions and behaviors - Self-control problems violate the rights of others and/or bring the individual into conflict with societal norms or authority figures Symptoms of these disorders can occur to a lesser degree in normally developing individuals (ie. Oppositional behavior) Before diagnosing a disorder, consider the frequency, persistence, pervasiveness across situations, and functional impairment relative to		Behaviors do NOT result in a serious violation of the rights of others, but problematic nonetheless			
	Conduct Disorder (CD)	- Repeated and persistent <i>serious</i> violation of rights/societal norms At least 3 symptoms occurring in OR across any of the four categories: Aggressive conduct: Bullies, uses weapons, cruelty to people/animals, rape, stolen with confrontation Deliberate property destruction: by fire or other means Deceit or theft: Broken in, lies, stolen without confronting Serious violation of rules: breaking curfew, running away, truancy If 18 years or older and criteria are not met for ASPD, conduct disorder can be dx'd		- Anger management & communication skills - Parental Training <i>Variable outcome</i> (concern is that behaviors escalate over time: ODD → CD → ASPD)	Subtypes of CD Childhood-onset Type (<10yrs) - Usually boys - Characterized by aggressive conduct - Often have a history of ODD and are at higher risk of ASPD Adolescent-onset type (10 years or older) - Less of a bias towards boys - Characterized by “serious violation of rules” (not “aggressive conduct”) <i>Conduct disorder can be diagnosed in an adult if ASPD criteria are not met</i> Etiology: Multifactorial causes, possible including: - Child’s constitutional temperament - Suboptimal parenting techniques (inconsistent/inappropriate responses to undesirable behaviors)
	Intermittent Explosive Disorder (IED)	Recurrent outbursts representing a failure to control aggressive impulses as manifested by either: Verbal aggression or non-damaging physical aggression occurring <i>frequently</i> OR			Typical Description of Act: - Described as brief “spells” - Precipitated by feeling frustrated - Usually self-reproach afterwards - Social, occupational, legal consequences

normative standards		- Damaging physical aggression occurring <i>infrequently</i> Aggression is: - Grossly disproportionate to stressor - Impulsive and/or anger-based (not instrumental) - Not explained by other causes/disorders			Differentials Conduct Disorder (CD) and ASPD - CD and ASPD are characterized by habitual, pervasive and instrumental (not impulsive) antisocial behavior - Those with IED are usually not aggressive and don't violate rights in-between explosive episodes Adjustment D.O. with Disturbance of Conduct (violation of rules) - Disturbed conduct begins after a causal psychosocial stressor (not just minor frustration), which can't be explained by another disorder Disruptive Mood Dysregulation Disorder (DMDD): - DMDD has severe temper outbursts disproportional to stressor but <i>with</i> irritable baseline mood
	Kleptomania	- Recurrent irresistible stealing of unneeded objects - Increasing tension before stealing - Pleasure, gratification, or relief when stealing - No other cause or motivating factor for theft <i>Stolen items are unneeded and typically of low value; they are hoarded, given away, or returned</i>		Impulse Control Problems: Behavioral Therapy Techniques - Avoiding triggers of behavior or substituting a behavior when triggers are encountered - Associate impulse with something noxious (aversion therapy) - Expose person to trigger and don't let impulsive action occur (Exposure & Repsonse Prevention)	Differential: "ordinary" shop lifting: - Object is the goal (not the process) - Usually planned and assisted
	Pyromania	Multiple episodes of deliberate fire setting with preceding tension or emotional arousal		Medication	Differential: Arson (non-pyromaniac): Need to assess motivation for setting fire

		- Fascination with fire and fire paraphernalia - Pleasure, gratification, or relief when setting fires or witnessing/participating in the aftermath - No other cause or motivating factor		Many types of medication have been tried (ie. SSRIs with variable success)	Biochemical correlates: ↓ serotonin associated with poor prefrontal impulse control ↑ dopamine associated with rewarding sensation during impulsive act (stimulation of brain’s reward pathway) - Ventral tegmental area to nucleus accumbens																					
Intellectual Disability	Intellectual Disability	- Deficits in intellectual functions: confirmed by clinical assessment and individualized, standardized, intelligence testing AND - Deficits in adaptive functioning in conceptual, social, and practical domains, that result in failure to meet developmental and sociocultural standards for personal independence and social responsibility. The deficits limit functioning in one or more activities of daily life (i.e. communication, social participation and/or independent living) – across multiple environments (home, school, work, community) AND - The onset is during the developmental period			Measurement of intellectual function - Standardized intelligence tests - Usually those developed by Wechsler <table><tr><td></td><td>Mean</td><td>Standard Deviation</td></tr><tr><td>Wechsler Adult Intelligence Scale (WAIS)</td><td>100</td><td>15</td></tr><tr><td>Wechsler Intelligence Scale for Children (WISC)</td><td>100</td><td>15</td></tr></table> ➤ For a diagnosis of intellectual disability, scores must be at least 2 Standard Deviations below mean (i.e., IQ < 70 +/- 5 for measurement error) Levels of Severity - Mild, moderate, severe, profound - Based on adaptive functioning (conceptual, social, practical), not IQ score <table><tr><td></td><td>Conceptual</td><td>Social</td><td>Practical</td></tr><tr><td>MILD (~ 85%)</td><td>Difficulties in learning – more support and more time needed; concrete approach to problems</td><td>Immature in social interactions and judgments</td><td>Age appropriate in personal care; needs support in complex tasks</td></tr><tr><td>MODERATE (~10%)</td><td>Marked lag in cognitive skills; ongoing assistance needed for conceptual tasks of day-to-day life</td><td>Communication and social limitations affect relationships; significant support needed in work settings</td><td>Extended teaching needed to care for personal needs; ongoing support needed to manage work and life</td></tr></table>		Mean	Standard Deviation	Wechsler Adult Intelligence Scale (WAIS)	100	15	Wechsler Intelligence Scale for Children (WISC)	100	15		Conceptual	Social	Practical	MILD (~ 85%)	Difficulties in learning – more support and more time needed; concrete approach to problems	Immature in social interactions and judgments	Age appropriate in personal care; needs support in complex tasks	MODERATE (~10%)	Marked lag in cognitive skills; ongoing assistance needed for conceptual tasks of day-to-day life	Communication and social limitations affect relationships; significant support needed in work settings	Extended teaching needed to care for personal needs; ongoing support needed to manage work and life
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MODERATE (~10%)	Marked lag in cognitive skills; ongoing assistance needed for conceptual tasks of day-to-day life	Communication and social limitations affect relationships; significant support needed in work settings	Extended teaching needed to care for personal needs; ongoing support needed to manage work and life																							

						<table><tr><th></th><th>Conceptual</th><th>Social</th><th>Practical</th></tr><tr><td>SEVERE (~ 3-4%)</td><td>Extensive supports for problem-solving throughout life; very little understanding of written language or numerical concepts</td><td>Limited spoken language; understands simple speech and gestures</td><td>Support and supervision required for all daily living activities; some maladaptive behavior including self-injury may be present</td></tr><tr><td>PROFOUND (~ 1-2%)</td><td>No understanding of symbolic processes; co-occurring motor or sensory impairments limit functioning</td><td>Communication is largely nonverbal and non-symbolic; may respond to social interactions through gestural and emotional cues; co-occurring deficits limit functioning</td><td>Dependent on others for all aspects of daily life</td></tr></table>		Conceptual	Social	Practical	SEVERE (~ 3-4%)	Extensive supports for problem-solving throughout life; very little understanding of written language or numerical concepts	Limited spoken language; understands simple speech and gestures	Support and supervision required for all daily living activities; some maladaptive behavior including self-injury may be present	PROFOUND (~ 1-2%)	No understanding of symbolic processes; co-occurring motor or sensory impairments limit functioning	Communication is largely nonverbal and non-symbolic; may respond to social interactions through gestural and emotional cues; co-occurring deficits limit functioning	Dependent on others for all aspects of daily life
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	Global Developmental Delay	- Child fails to meet expected developmental milestones in several areas of intellectual (cognitive) functioning - Clinical severity level cannot be reliably assessed Diagnosis reserved for children under the age of 5 - Reassessment required after a period of time, if possible	Must be under the age of 5															
Autism Spectrum Disorder	Autism Spectrum Disorder	A. Persistent deficits in social communication and social interaction across multiple contexts, as manifested by - Deficits in social-emotional reciprocity - Deficits in nonverbal communicative behaviors used for social interaction - Deficits in developing, maintaining, and understanding relationships AND B. Restricted, repetitive patterns of behaviors (RRBs) interests, or activities, as manifested by 2 or more of the following: - Stereotyped or repetitive motor movements, use of objects, or speech - Insistence on sameness, inflexible adherence to routines, or ritualized patterns of verbal or nonverbal behavior - Highly restricted, fixated interests that are abnormal in intensity or focus - Hyper – or hypo reactivity to sensory input or unusual interest in sensory aspects of the environment (usually			Specifiers - With or without accompanying intellectual impairment - With or without accompanying language impairment - Associated with a known medical, genetic, or environmental factor - Associated with another neurodevelopmental, mental, or behavioral disorder <i>*Asperger’s syndrome (former DSM diagnosis) = “without accompanying intellectual impairment and without accompanying language impairment”</i> Severity - Rating from level 1 (least severe) to level 3 (most severe) - Separate ratings for social functioning level and for RRBs - Ratings based on functional impairment and needed support													

		hyper) AND C. Symptoms must be present in the early developmental period (but may not become fully manifest until social demands exceed limited capacities or may be masked by learning strategies later in life) (Usually diagnosed by age 2) AND D. Symptoms cause clinically significant impairment in social, occupational, or other important areas of current functioning AND E. Not better accounted for by intellectual disability			
Communication Disorders	Language Disorder	A. Persistent difficulties in the acquisition and use of language across modalities (spoken, written, sign, etc.) • Reduced vocabulary • Limited sentence structure • Impairments in discourse AND B. Language abilities substantially below age expectations AND C. Onset in the early developmental period AND D. Not attributable to sensory impairment, motor dysfunction, other medical or neurological condition, and not better explained by intellectual disability or global developmental delay			Diagnoses made before the age of 4 may be unreliable

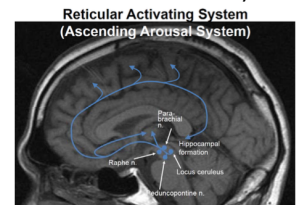
	Speech-sound Disorder	A. Persistent difficulty with speech sound production that interferes with speech intelligibility or prevents verbal communication or messages AND B. The disturbance causes limitations in effective communication that interferes with social, academic, occupational performance AND C. Onset of symptoms in the early developmental period AND D. Not attributable to congenital or acquired conditions, other medical or neurological conditions, hearing loss, or brain injury			Overall, speech should be mostly intelligible at age 3-4
	Social (pragmatic) Communication Disorder	A. Persistent difficulties in the social use of verbal and nonverbal communication (all of the following) • Deficits in using communication for social purposes (greeting, sharing info) • Impairment in ability to change communication to match context • Difficulty following the rules for communication • Difficulty understanding implicit meanings AND B. The deficits result in functional limitations in effective communication that interfere with social, academic, occupational performance AND			Diagnosis is rarely made before the age of 4-5 *Similar to Autism but Autism has RRBs

		C. Onset in the early developmental period AND D. Not attributable to low abilities in word structure and grammar or to other medical, neurological, or mental disorders			
	Childhood-onset Fluency Disorder	A. Persistent disturbance in the normal fluency and time patterning of speech that are inappropriate for age and level of language skill • Sound and syllable repetitions • Sound prolongations of consonants and vowels • Broken words • Blocking – pauses in speech • Words produced with excessive tension • Monosyllabic whole-word repetitions (i.e. “I-I-I-I see him”) AND B. The disturbance causes anxiety about speaking or limits effective communication AND C. Onset of symptoms in the early developmental period (Note: adult-onset fluency disorder) AND D. Not attributable to speech-motor or sensory deficit, dysfluency associated with a neurological insult, other medical conditions, and not better explained by another mental disorder			• Usually diagnosed by age 6 • 65-85% of children recover from the dysfluency • The dysfluency is often absent during oral reading, singing, or talking to inanimate objects or pets
Specific Learning Disorder	Specific Learning Disorder	A. Difficulty learning and using academic skills, as indicated by the presence of at least one of	At least 6 months		Requires a psychoeducational assessment

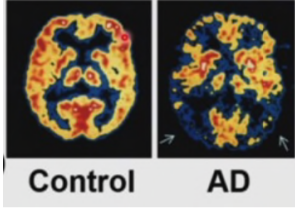
		the following symptoms persisting for at least 6 months, despite targeted interventions • Inaccurate or slow and effortful word reading • Difficulty understanding the meanings of what is read • Difficulties with spelling • Difficulties with written expression • Difficulties mastering number sense, facts, or calculation • Difficulties with mathematical reasoning AND B. The affected academic skills are: • Substantially and quantifiably below those expected for the individual's chronological age • Cause significant interference with academic or occupational functioning or activities of daily living • Confirmed by individually administered achievement measures and comprehensive clinical assessment AND C. The learning difficulties began during school-age years but may not be fully manifest until the academic demands exceed capacity AND D. Not better accounted for by intellectual disabilities, impaired sensory abilities, lack of proficiency in the language of instruction, inadequate instruction, or other mental or neurological disorders			Specifiers • With impairment in reading • With impairment in written expression • With impairment in mathematics Severity: • Mild: Mild difficulties in 1 or 2 academic domains; may learn to function well with initial support services • Moderate: Marked difficulties in 1 or 2 academic domains; needs accommodations/support to complete tasks • Severe: Severe difficulties across several academic domains; needs intensive and individualized teaching
Attention-Deficit/ Hyperactivity Disorder	ADHD	A. A persistent pattern of inattention and or/hyperactivity-impulsivity that interferes with functioning or development, as characterized by (1) or (2)	At least 6 months	A combination of medication and behavioral psychotherapy are recommended (if child <6 years old, then behavioral interventions are first-line)	Subtypes • ADHD, combined presentation (both criteria A1 and A2 are met for the past 6 months)

		(1) Inattention: 6 or more specific symptoms of inattention have persisted for a least 6 months to a degree that is consistent with developmental level, and that negatively impacts directly on social and academic/occupational activities 1. Lack of attention to details 2. Difficulty in remaining focused 3. May seem not to be listening 4. Lack of follow through 5. Difficulty organizing tasks 6. Avoids tasks that requires sustained effort 7. Loses things 8. Easily distracted 9. Forgetful (2) Hyperactivity and Impulsivity: 6 or more specific symptoms of hyperactivity-impulsivity have persisted for at least 6 months to a degree that is inconsistent with developmental level and that negatively impacts directly on social and academic/occupational functioning 1. Fidgets, squirms in seat 2. Leaves seat, when being seated is expected 3. Runs about, inappropriately (adult may feel restless) 4. Unable to remain quiet while at play or leisure 5. On-the-go; motor driven 6. May talk excessively 7. Blurts out, cannot wait for turn 8. Interrupts, intrudes	Stimulant Medications: Methylphenidate (Ritalin, Concerta, Quillivant) • Mechanism of action: increases dopamine • Adverse side effects: appetite loss, insomnia, edginess, GI upset • Black box warning: potential abuse Selective Norepinephrine Reuptake Inhibitor Drugs Atomoxetine (Strattera) • Used in stimulant failures, or if there are contraindications for taking stimulants (i.e. co-existing anxiety, tics, abuse liability)	• ADHD, predominantly inattentive (criteria A1 met for the past 6 months) • ADHD, predominately hyperactive-impulsive (criteria A2 met for the past 6 months) Severity is specified: Mild, Moderate, Severe
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		B. Some yperactive-impulsive or inattentive symptoms that caused impairment were present before age 12 years C. Some impairment from the symptoms is present in two or more settings (i.e. at school, work, home) D. There msut be clear evidence of clinically significant impairment in social, academic, or occupational functioning E. The symptoms do not occur exclusively during the course of another mental disorder			
Neurocognitive Disorders	Delirium	- Disturbance in awareness and attention Awareness: assessed by one's orientation to the environment Attention: assessed by one's ability to direct, focus, sustain, and shift attention An additional disturbance in a cognitive domain (i.e. Memory, language, thoughts [delusions], and perceptions [including hallucinations, illusions]) Sudden onset of symptoms (over hours to a few days) that fluctuate (waxes and wanes) during the day Evidence for a direct physiological cause (i.e. A medical condition, drug intoxication/withdrawal)		Treat underlying medical condition Manage associated symptoms (i.e. agitation, psychosis) - Use antipsychotics to treat associated symptoms (i.e. agitation, psychosis) of most deliriums - Use benzodiazepines to treat delirium caused by <u>alcohol withdrawal</u> Utilize environmental supportive measures - Regulate amount of environmental stimulation - Provide orienting stimuli (i.e. lighting, personal effects, sensory aids) - Provide for safety needs (i.e. attendant, bedrails, and possibly restraints)	Pathology - Multiple etiologies (i.e. fever, dehydration) - Widespread brain regions affected - Core deficits in central cholinergic functioning - Deficits in the reticular activating system and its ascending connections (important for attention and arousal) Risk Factors Non-modifiable (i.e. poor health, older age, male gender) – <i>not sure why gender bias exists</i>



					• Modifiable (i.e. immobilization, poor sleep, and use of benzodiazepines in an ICU) Course • Symptoms persist until cause is reversed • Resolution typically occurs within 3-7 days • Amnesia for events during delirium is common • Delirium is a poor prognostic sign for long-term survival and results in longer ICU stays
	Dementia	• Multiple and severe cognitive impairment without impaired consciousness • Usually progressive and irreversible • Most commonly occurs in the elderly			“Mild cognitive Impairment” (MCI) – Refers to cognitive decline that doesn’t cause impairment in activities of daily living. Not sure what staves off progression to dementia.
	Alzheimer’s Dementia (AD)	• Significant memory impairment plus impairment in at least 1 other cognitive domain • A gradual onset with steadily progressive decline • Exclusion of other causes of the symptoms (i.e. stroke) Neuropathology of AD • Neuroanatomical ○ Cortical atrophy ○ Hippocampal atrophy ○ Enlarged ventricles • Neurochemical ○ Multiple neurotransmitter deficiencies		3 Cholinesterase inhibitors (approved by the FDA) • Donepezil (Aricept) • Galantamine (Razadyne) • Rivastigmine (Exelon) 1 NMDA receptor blocker (approved by the FDA) • Memantine (Namenda) <i>There is a combination drug of memantine and donepezil (Namzaric)</i> • AD drugs are considered “ineffective”	General Course of AD Onset typically in late 70’s with 10+ year progression Early Stages: • Memory deficits (rapid forgetting) • Anomia (word-finding difficulties) Middle Stages • Further memory and language decline • Visuospatial deficits (getting lost) • Agnosias (loss of knowledge of a perception) ○ Visual agnosias (visual stimuli are perceived but not identified)

		○ Focus has been on loss of cholinergic (ACh) neurons in the nucleus basalis of Meynert due to its role in memory formation • Neurofunctional:  posterior hypometabolism (parietal/temporal) • Histopathological: B-amyloid plaques and neurofibrillary tangles In vivo Biomarkers of AD: ○ CSF Amyloid & tau levels ○ PET imaging of amyloid plaques <i>Definitive AD diagnosis still depends on post-mortem histopathological confirmation</i>	• Side effects (i.e. hypotension, GI) have notable consequences for the elderly (i.e. risk of falls) • Research efforts are on decreased production and increased clearance of B-amyloid through antibody drugs Other interventions for AD • Treatment of neuropsychiatric symptoms (i.e. aggression, agitation) ○ Non-pharm methods (music therapy?) ○ Antipsychotics (used off-label for demented patients. FDA black-box warning of increased mortality risk of used with this population) ○ Anticonvulsants & antidepressants (select ones) • Rule out non-dementia causes of behavioral problems (things like pain/discomfort) in AD patients that can't express these issues Psychosocial considerations • Use of external mnemonics in early AD stages • Assessment and restriction of driving • Safe return program for wanderers Support for caregivers	○ Prosopagnosia: specific inability to identify familiar faces by sight ○ Auditory agnosias (sounds perceived but not identified – can't tell that a phone ringing means to pick it up) ○ Anosognosia: lack of awareness of one's disability – forget that they're forgetting • Mood changes • Personality changes (formerly gregarious, now reclusive) • Psychosis (hallucinations, paranoid delusions) Late Stages • Severe deficits in all cognitive domains • Global aphasia • Motor dysfunction • Death from opportunistic infections
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				• Risk for care giver syndrome from psychological, social, and financial sequelae of caregiving • Resources through AD Association/AD Foundation: ○ Support groups ○ Caregiver tips (i.e. how to respond to psychosis) ○ Long-term care options ○ Legal and financial planning	
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<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Panic Disorder	Recurrent and unexpected panic attacks that exhibits fear of future attacks (or consequences of them) or a significant change in behavior related to the attacks (needs an escort)	≥ 1 month	Can experience panic attacks with somatic or cognitive symptoms like chest pain or shortness of breath	• CBT (Cognitive Behavioral Therapy) • Anxiolytics
Agoraphobia	≥ 2 of these situations are feared/avoided: • Using public transportation • Being in open spaces • Being in enclosed spaces • Standing in line or being in a crowd • Being outside the home alone	Unspecified	These situations are feared/avoided because of thoughts of escape might be difficult if incapacitating or embarrassing symptoms develop Can develop in Panic Disorder patients	• CBT • Anxiolytics
Specific Phobia	An excessive, persistent, and disproportional fear of a specific object or situation	Unspecified	Can be animals, natural environment, situational, blood-injection injury, and etc.	• CBT • Anxiolytics
Social Anxiety Disorder	Excessive, persistent, and unrealistic fear of social situation involving possible scrutiny by others due to fear of negative evaluation	Unspecified	Formerly “social phobia” Needs to be specified “Performance only” if fear is restricted to speaking or performing in public	• CBT • Anxiolytics
General Anxiety Disorder (GAD)	Persistent uncontrolled anxiety about multiple events	≥ 6 months	Accompanied by symptoms like: • Restless, on edge • Decreased concentration • Muscle tension • Fatigued • Irritability • Insomnia	• CBT • Anxiolytics
Separation Anxiety Disorder	Developmentally inappropriate and excessive anxiety concerning separation from attachment figures	Unspecified	A person (usually child) experiences marked distress about: • Anticipated/actual separation • Harm to attachment figure • Going out (school/sleepover)	• CBT • Anxiolytics

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Selective Mutism	Refusal to speak in specific situation despite fluent speech in other contexts	≥ 1 month (not the 1 st month of school)	Written communication and social play may also be affected This is not due to a lack of language knowledge or a communication disorder (stuttering)	• CBT • Anxiolytics
OCD	Recurrent obsessions (increase distress) or compulsions (decrease distress) Obsessions/compulsions are time consuming or distressful/disruptive Symptoms are not explained by another disorder	Unspecified	Some individuals will be “with absent insight (delusional beliefs)” where they’re completely convinced their OCD beliefs are true Etiology due to cortico-striato-thalamo-cortical (CSTC) overactivity and serotonin deficiency Can also be caused by <i>PANDAS</i> (after streptococcal infection) Can be assessed with Yale-Brown Obsessive Compulsive Scale (Y-BOCS)	• Behavioral Therapy (exposure and response) • Antidepressant (SSRIs) • Psychosurgery (cingulotomy or capsulotomy) • Deep Brain Stimulation • Antibiotics (PANDAS)
Hoarding Disorder	Difficulty paring with possessions due to perceived need to save items or distress about discarding them Hoarding causes distress/impairment and is not better explained by another disorder	Unspecified	Needs to be specified if “with absent insight (delusional beliefs)”	• CBT
Body Dysmorphic Disorder (BDD)	Preoccupation with a perceived flaw in physical appearance (flaw is minimal or non-observable) that causes functional impairment Repetitive behaviors/mental acts are performed in response to appearance concerns	Unspecified	Preoccupation is not better accounted for by an eating disorder Needs to be specified “with muscle dysphoria” or “with absent insight (delusional beliefs)”	• CBT

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Excoriation Disorder	Recurrent skin picking resulting in skin lesions with attempts to stop picking Results in distress/impairment	Unspecified	Not better explained by another disorder	• CBT
Trichotillomania	Recurrent pulling out of one's hair resulting in hair loss with attempts to stop hair pulling Results in distress/impairment	Unspecified	Not better explained by another disorder	• CBT • Urge-distraction techniques (spinner ring)
Posttraumatic Stress Disorder (PTSD)	Exposure to a traumatic stressor: • Actual or threatened death, serious injury, or sexual violence • Exposure which may include direct experience, witnessing event, or learning of event to close family member/friend ≥ 1 symptoms from each of the 4 categories • Intrusion - Dreams - Recollections - Feeling event reoccur - Psychological or Physiological distress when encounters symbols • Avoidance (reminders of events) - Thoughts - Places - Conversations • Negative Alterations in Cognition/Mood - Negative beliefs/expectations - Negative Emotional States - Inability to experience positive emotion - Diminished interest/participation - Detachments/estrangement - Dissociative Amnesia • Alterations in Arousal/Reactivity - Sleep disturbance - Irritability and angry outbursts - Reckless/Self-destructive behavior - Concentration problems - Hypervigilance - Exaggerated startle response	> 1 month	Symptoms usually begin within 3 months of the trauma BUT can begin anytime Very vulnerable to young adults Can also happen to patient with sudden-onset medical events (heart attack) Hyperresponsive amygdala Underresponsive prefrontal cortex Reduced volume and dysfunction of hippocampus	• Psychotherapy • Medications (antidepressants & benzodiazepines)

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Acute Stress Disorder (ASD)	Exposure to a traumatic stressor (actual threatened death, serious injury, or sexual violence) Numerous PTSD-like symptoms must develop from the four PTSD categories	3 days – 1 month after trauma exposure	Hyperresponsive amygdala Underresponsive prefrontal cortex Reduced volume and dysfunction of hippocampus	• Psychotherapy • Medications (antidepressants & benzodiazepines)
Adjustment Disorder	Development of significant (and disproportional) emotional/ behavioral symptoms due to an identifiable stressor (often an ordinary life experience - retirement) Symptoms develop within a few months of stressor onset but are expected to resolve within a several months after the stressor NOT explained by another disorder	Unspecified	Can be “with depressed mood”, “with anxiety”, or “with disturbance of conduct” (vandalizing) Considered a “<u>last resort</u>” <u>diagnosis</u> in that only if there’s a causal stressor and no other disorder that explains the symptoms.	• Learning coping strategies in psychotherapy and in support groups • May involve crisis intervention like hospitalization or medications
Reactive Attachment Disorder (RAD)	Inhibited and emotionally-withdrawn behavior towards adult caregivers (grossly underdeveloped attachment)	Unspecified	Usually seen with children Considered a direct result of pathogenic (basic disregard) care early in life	• CBT
Disinhibited Social Engagement Disorder (DSED)	Overly familiar behavior with relative strangers (indiscriminate social behavior)	Unspecified	Usually seen with children Considered a direct result of pathogenic (basic disregard) care early in life	• CBT

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Dissociative Amnesia	Memory loss for autobiographical information which doesn't occur due to another disorder Memory loss can be: • Localized: total loss of memory during a certain period • Selective: some (but limited) recall of personal memories during a certain period of time • Generalized: loss of personal memory of <u>entire life</u> up to and including event	Unspecified	Can be specified "with dissociative fugue" • Purposeful travel or wandering with amnesia for identity or other autobiographical information • Usually hours to days • Unobtrusive lifestyle • Spontaneous termination of amnesia	• CBT • Hypnosis
Dissociative Identity Disorder (DID)	Disruption of identity characterized by ≥ 2 distinct personality states • The Primary (host) • An Alter Inability to recall personal information (as evidenced by frequent memory gaps in the host when the alter took over)	Unspecified	Formerly known as Multiple Personality Disorder Patients may also experience unexpected changes in personal possessions	• CBT • Hypnosis
Depersonalization or Derealization Disorder	Either or both: - Depersonalization: experiences of unreality, detachment, or being an outside observer <u>with respect to one's thoughts, feeling, sensations, body, or actions</u> (feeling numb) - Derealization: experiences of unreality or detachment <u>with respect to surroundings</u> (seems dreamlike)	Unspecified	Reality testing would remain intact (person knows that the perceptual experience is just a misperception) Would see functional impairment Drugs/diseases must be eliminated as causes	• CBT • Hypnosis

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Major Depressive Disorder (MDD)	A person must experience ≥ 1 major depressive episode (MDE) for a diagnosis of MDD An MDE is at least 2 weeks of ≥ 5 of the following including at least 1 of the required symptoms: • <u>Depressed mood</u> • <u>Anhedonia (inability to feel pleasure)</u> • Significant weight change • Insomnia/Hypersomnia • Loss of energy • Psychomotor changes • Feeling guilty/worthless • Decreased concentration • Thoughts of death/suicide ideation The person can't have a history of mania or hypomania	> 2 weeks (MDE)	Can be “with melancholic features” where the patient has: • Worse depression in the morning • Early-morning awakenings • Significant appetite loss Can be “with atypical features” where the patient has: • Mood reactivity • Weight/sleep increase • Lead paralysis Can be “with mood-congruent psychotic features” • Content of delusions or hallucinations are depressive Can be “with mood-incongruent psychotic features” • Content of delusions or hallucinations are not depressive Can be “with catatonia” • Mutism • Immobility • Waxy flexibility Can be “with anxious distress” • MDE with feeling tense, restless, and fearful Can be “with peripartum onset” • During pregnancy or within 4-week post delivery Can be “with seasonal pattern” • Consistent temporal relation with time of year and MDEs	• CBT • Antidepressants (monoamines) • Electroconvulsive Therapy (ECT) • Transcranial Magnetic Stimulation (TMS) • Vagus Nerve Stimulation (VNS)

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Persistent Depressive Disorder (PDD)	Chronic depressed mood for ≥ 2 years Can be in in the form of: • A long-lasting MDE or • Dysthymia (≥2 of the following) - Poor appetite/overeating - Insomnia/Hypersomnia - Low energy - Low self-esteem - Trouble concentrating or making decisions - Feelings of hopelessness	≥ 2 years	Can be “with pure dysthymic syndrome” • Prolonged dysthymia but <u>no MDE occurs</u> Can be “with persistent MDE” • Prolonged MDE Can be “with intermittent MDEs” • Dysthymic periods and MDEs Mood can be “euthymic” beyond two years and is still considered PDD	Only for Dysthymic • CBT • Antidepressants (monoamines) • Electroconvulsive Therapy (ECT) • Transcranial Magnetic Stimulation (TMS) • Vagus Nerve Stimulation (VNS)
Premenstrual Dysphoric Disorder (PMDD)	Mood lability, irritability, dysphoria, and anxiety Must be present in the week before menses onset, improve a few days after menses onset, and minimizes in the week post-menses Must cause clinically significant distress or functional impairment	Unspecified	Validated clinical scales are used to confirm the diagnosis	• Several different SSRIs that are taken intermittently during a month or even continuously
Disruptive Mood Dysregulation Disorder (DMDD)	Severe temper outbursts at least 3 times/week Sad, irritable, or angry mood almost daily Reaction is disproportionate to situation Symptoms are present in multiple settings Child must be at least 6 years old Symptoms must begin before age 10	3 episodes per week	Usually in children Questionable valid diagnostic entity, basically made so child won't be labeled as bipolar	• CBT

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Bipolar I Disorder (BPI)	Must experience at least 1 <u>manic</u> episode: Abnormally elevated mood (or irritability) PLUS increased energy AND ≥ 3 additional symptoms (DIGFAST): • Distractibility • Irresponsibility • Grandiosity • Flight of ideas, racing thoughts • ↑ Activity (goal-directed) • ↓ Sleep need • Talkativeness (pressured speech) Also known as “<i>Manic Depression</i>”	≥ 1 Week	BPI is more genetically-controlled than MDD Many patients will experience MDE's but this is not required for the diagnosis of BPI Can be “with rapid cycling” • If patient experiences ≥ 4 mood episodes per year Can be “with psychotic features” • If delusion or hallucinations are present • <u>Specify if mood-congruent or mood-incongruent</u>	• Mood stabilizers - Manic only - Lithium - Anticonvulsants - Antipsychotics • Bipolar Treatment - Antidepressants (not FDA approved for BP depression because it can induce mania) - Antipsychotics (only FDA approved) • ECT (#1 use is BPI) • Psychotherapy (used for <u>secondary</u> issues)
Bipolar II Disorder (BPII)	Must experience <u>at least 1 MDE</u> and at least 1 <u>hypomanic episode</u> Hypomania includes the same symptoms of mania but of different severity and must last ≥ 4 days: <u>Mania causes marked impairment in function whereas hypomania does not</u>	≥ 4 days of hypomania + MDE (2 weeks)	The more disruptive problem in in BPII disorder is the depressed phase, hypomania does not significantly affect the patient's life	• Same as for BPI but with a focus on the use of bipolar depressive drugs (no risk of inducing mania)
Cyclothymic Disorder	For ≥ 2 years, a person experiences periods of hypomanic symptoms that fluctuate with periods of depressive symptoms Criteria for a MDE <u>have never been met</u>	≥ 2 years	Similar to BPI but the mood shifts are not as extreme The patient may ultimately have an MDE or manic episode which will change the diagnosis	• Mood Stabilizers (as for BPI)

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Schizophrenia	An Active-Phase for ≥ 1 month which is defined as: At least 2 psychotic domain symptoms with at least <u>one being a core symptom</u> <u>Delusions</u> - A fixed belief that is not amenable to change - Can be <i>grandeur, persecution, control, reference, or thought broadcasting</i> <u>Hallucinations</u> - A sensory perception without external stimulus - Can be <i>auditory</i> which is usually derogatory <u>Disorganized Speech</u> - Tangentiality: shifting to slightly related topic - Flight of ideas: extreme tangentiality (rapid) - Derailment: no connections with ideas - Clang Association: words based on rhyming - Word Salad: no relationship with words (clear) - Grossly disorganized behavior - Disorganized: non-goal orientated behavior - Catatonia: posturing, waxy flexibility, mutism - Negative Symptoms - Diminished emotional expression - Avolition (lack of motivation) - Alogia (diminished speech) - Anhedonia (lack of interest) - Asociality (disinterest in social interactions) At least 1 symptom persists for ≥ 6 months	Active Phase for ≥ 1 month + 1 symptom for ≥ 6 months	No subtypes except to specify “with catatonia” Usually associated with low socioeconomic attainment (downward drift), nicotine addiction, and suicide risk with aggressive tendencies Usually onset in late teens with later onset in women A late onset is most likely female with positive symptoms and less severe course Childhood onset is a severe form with notable neurodegeneration It’s hypothesized that overactivity of mesolimbic DA relates to positive symptoms and <u>underactivity of mesocortical DA relates to negative symptoms</u> Associated with lateral ventriculomegaly, cortical and hippocampal atrophy (with cellular disorganization), decreased volume of thalamus, and hypofrontality (affects perseverance)	- Antipsychotics are the mainstay treatment - All are D2 receptor antagonists - But can lead to EPS - Can also cause NMS which can lead to death - Also atypical antipsychotics which decrease risk of EPS - But present 1-2% risk of agranulocytosis ($\downarrow\downarrow\downarrow$ WBCs) - Supportive Interventions - ECT Augmentation (only for refractory schizophrenia) - Psychosurgery (lobotomy) is no longer used

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Schizophreniform Disorder	At least 1 month of Active-Phase symptoms: At least 2 psychotic domain symptoms with at least <u>one being a core symptom</u> • <u>Delusions</u> • <u>Hallucinations</u> • <u>Disorganized Speech</u> • Grossly disorganized behavior • Negative Symptoms Signs of the disturbances last LESS than 6 months	Active Phase for ≥ 1 month + symptom for < 6 months	Basically schizophrenia before 6 months <u>There is no such thing as provisional schizophrenia</u>	• Schizophrenia treatments
Brief Psychotic Disorder	At least 1 <u>core symptom</u> that last < 1 month • <u>Delusions</u> • <u>Hallucinations</u> • <u>Disorganized Speech</u>	< 1 month	Patient returns to full premorbid functioning Specified “with marked stressor” or “without marked stressor”	• CBT
Delusional Disorder	Presence of delusion for at least ≥ 1 month with no other psychotic domain symptoms There is no functional impairment aside from the direct impact of the delusion	≥ 1 month	Delusion can develop in context of a close relationship with psychotic individual (formerly “Shared Psychotic Disorder”) Can be “with bizarre content” 4 Subtypes: • Persecutory • Grandiose • Erotomaniac • Jealous • Somatic (fix my cancer!) • Unspecified	• Antipsychotics • Trial Separation (if shared) • Increasing sources of reality testing

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Schizoaffective Disorder	Active phase of schizophrenia occurs concurrently with a major mood episode (a MDE or manic episode) The active phase must last ≥ 2 weeks without mood symptoms Mood symptoms need to be present for “<u>the majority of the total duration</u>”	Active Phase (≥ 1 month) & MDE/Manic Episode (for majority) + Active Phase (alone ≥ 2 weeks)	Subtypes: • <u>Bipolar</u> (if mania is experienced) • <u>Depressive</u> (if depression is experienced but not mania)	• Antipsychotics • Supportive Interventions
Somatic Symptom Disorder (SSD)	≥ 1 distressing/disruptive somatic symptom with excessive thoughts/feelings about the symptoms: • Disproportionate thoughts to the seriousness • High levels of anxiety • Excessive energy/time devoted to the symptom The patient COULD have a medical explanation for the symptoms and still have the disorder	Unspecified	Different between SSD and delusional disorder is the intensity which is far greater with delusions Sometimes due to overactivity of key brain regions that are involved in the “unpleasantness” of bodily sensations or cognitive biases May be due to behavioral consequences	• CBT • To reduce stress • To reduce excessive attention • To correct cognitive distortions • To reinforce “non-sick role”
Illness Anxiety Disorder (IAD)	Preoccupation with having/acquiring a serious illness even though somatic symptoms are extremely mild (stomach ache after a meal) or not present Patients will perform excessive health-related behaviors or maladaptively avoid health care	Unspecified	Frequently seen with “doctor shopping” Sometimes due to overactivity of key brain regions that are involved in the “unpleasantness” of bodily sensations or cognitive biases May be due to behavioral consequences	• CBT • To reduce stress • To reduce excessive attention • To correct cognitive distortions • To reinforce “non-sick role”

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Conversion Disorder	Altered voluntary motor or sensory function with evidence of <u>incompatibility between the symptoms and neurological symptoms</u> Also known as <i>Functional Neurological Syndrome</i>	Unspecified	Typically, after a major stressor Subtypes: • With weakness or paralysis • With abnormal movement • With swallowing symptoms • With speech symptom • With attacks or seizures • With anesthesia or sensory loss • With special sensory symptom	• Psychotherapy
Factitious Disorder Imposed on Self	A patient feigns symptoms in oneself in the <u>absence</u> of obvious “ external ” rewards	Unspecified	Also referred to as “ <i>Munchausen’s Syndrome</i> ”	• NONE
Factitious Disorder Imposed on Another	A patient feigns symptoms in another person in the <u>absence</u> of obvious “ external ” rewards	Unspecified	Also referred to as “ <i>Munchausen’s Syndrome by Proxy</i> ” Usually mother -> child	• NONE • Be sure to report to Child Protective Services
Malingering	A patient feigns symptoms in oneself for “<u>external</u>” rewards	Unspecified	Common example is avoiding work	• Unspecified
Malingering by Proxy	A patient feigns symptoms in another individual for “<u>external</u>” rewards	Unspecified	Common examples are child disability	• Unspecified

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Anorexia Nervosa	<u>Restriction</u> of food that leads to being significantly underweight = BMI <17 or for <5th percentile for children 2-18 yrs Intense fear of weight gain Body image disturbance	> 3 months	Subtypes (> 3 months): • <u>Binge-eating/purging</u> for patients who binges and/OR purges • <u>Restricting</u> for patients who do NOT recurrently binge or purge A purge can be laxatives, vomiting, diuretics, enemas, or excessive exercise Can be associated with <u>hypercholesterolemia</u> (slow metabolism), lanugo (prenatal fuzzy hair), leukopenia, and etc. Comes with genetic susceptibility	• In-Patient Hospitalization • Psychotherapy
Bulimia Nervosa	<u>Recurrent binge eating</u> (large amount, out of control) Recurrent inappropriate compensatory behavior for binge	≥1/week for 3 months	No subtypes Typically normal or slightly overweight <u>Low/unstable serotonin is associated</u>	• CBT • Antidepressant for impulse control
Binge Eating Disorder (BED)	Binge eating for ≥1/week for 3 months The bingeing is <u>associated with</u>: • Rapid eating • Eating until uncomfortably full • Eating when not physically hungry • Eating alone due to shame about quantity • Feeling disgusted/guilt/depressed after binge <u>NO inappropriate compensatory behavior</u>	≥1/week for 3 months	Usually associated with obesity (BMI ≥30)	• CBT • Antidepressant for impulse control

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Intellectual Disability	Deficits in intellectual functions and adaptive functioning during the developmental period <u>Intelligence test has to be 2 standard deviations of the norm</u>	Unspecified	Level of severity is based on Adaptive Functioning (mild, moderate, severe, profound)	• Unspecified
Global Developmental Delay	Child fails to meet expected developmental milestones in <u>several areas</u> of intellectual (cognitive) functioning Clinical severity level cannot be reliably assessed Diagnosis <u>reserved for children under the age of 5</u>	Unspecified	A reassessment is required after a period of time, if possible	• Unspecified
Autism Spectrum Disorder	Persistent deficits in social communication and social interaction across multiple contexts as manifested by: - Deficits in social-emotional reciprocity - Deficits in nonverbal communicative behaviors used for social interaction - Deficits in developing, maintaining, and understanding relationships Restricted, repetitive patterns of behavior, interests, or activities as manifested by at least 2 of the following: - Stereotyped/repetitive motor movements, use of objects, or speech - Insistence on sameness - Highly restricted, fixated interests - Hyper/hypo reactivity to sensory input Symptoms must be present in the early developmental period Symptoms must cause clinically significant impairment in social, occupational, and other areas	Unspecified	Usually diagnosed by age 2 Level 1 – Some support needed Level 2 – Substantial support needed Level 3 – Very substantial support needed	• Unspecified

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Language Disorder	Persistent difficulties in the acquisition and use of language across modalities (spoken, written, sign, etc.) - Reduced vocabulary - Limited sentence structure - Impairments in discourse <u>Language abilities substantially below age expectations</u> Onset in the early developmental period and is NOT attributable to sensory impairment, motor dysfunction, or other medical condition	Unspecified	Diagnoses made before age of 4 may be unreliable	• Unspecified
Speech-Sound Disorder	Persistent difficulty with <u>speech sound production</u> that interferes with speech intelligibility or prevents verbal communication or messages The disturbance causes <u>limitations in effective communication</u> that interferes with social, academic, and occupational performance Onset in the early developmental period and is NOT attributable to congenital or acquired conditions.	Unspecified	Overall, speech should be mostly intelligible at age 3-4	• Unspecified
Social (Pragmatic) Communication Disorder	Persistent difficulties in the social use of verbal and nonverbal communication - Impairment in ability to change communication to match context - Difficulty in following the rules for communication - <u>Difficulty understanding implicit meanings (understanding jokes)</u> Onset in early development period and is not attributable to abilities in word structure, grammar or medical/neurological conditions.	Unspecified	Diagnoses is rarely made before the age of 4-5	• Unspecified

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Childhood-Onset Fluency Disorder (stuttering)	Persistent <u>disturbances in the normal fluency and time patterning of speech</u> that are inappropriate for age and level of language skill - Sound and syllable repetitions - Sound prolongations of consonants and vowels - Broken words - Blocking – pauses in speech - Words produced with excessive tension - Monosyllabic whole-word repetitions The disturbance <u>causes anxiety about speaking</u> or limits effective communication Onset in early development period and is not attributable to speech-motor or sensory deficit, dysfluency associated with neurologic insult, or other medical conditions	Unspecified	Different from adult-onset fluency disorder Usually diagnosed by 6 65-85% of children recover from the dysfluency The dysfluency is often absent during oral reading, singing, or talking to inanimate objects or pets	• Unspecified
Specific Learning Disorder	<u>Difficulty learning and using academic skills</u>, as indicated by the presence of <u>at least one of the following symptoms persisting for at least 6 months</u>, despite targeted interventions - Inaccurate or slow/effortful reading - Difficulty understanding the meaning of what is read - Difficulties with spelling - Difficulties with written expression - Difficulties mastering number sense, facts, or calculations - Difficulties with mathematical reasoning The affected academic skills are substantially and quantifiably below what's expected for age, cause significant interference with academic, occupational, and activities of daily living Onset in school age years but may not manifest until academic demands exceed capacity and are NOT better accounted for by intellectual disabilities, impaired intellectual/sensory abilities, lack of proficiency in the language of instruction, inadequate instruction, or other mental/neurologic disorders	≥1 symptom for ≥6 months	Requires psychoeducational assessment	• Unspecified

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Attention Deficit Hyperactivity Disorder (ADHD)	A persistent pattern of inattention and/or hyperactivity impulsivity that interferes with functioning or development, as characterized by (1) or (2): <u>Inattention: Six or more specific symptoms of inattention for at least 6 months</u> that is inconsistent with a developmental delay and that negatively impacts directly on social and academic/occupational activities: - Lack of attention to detail - Difficulty in remaining focused - May seem to not be listening - Lack of follow through - Difficulty organizing tasks - Avoids tasks that require sustained - Loses things - Easily Distracted - Forgetful <u>Hyperactivity and Impulsivity: Six or more specific symptoms of hyperactivity-impulsivity for at least 6 months</u> that is inconsistent with a developmental delay and that negatively impacts directly on social and academic/occupational functioning: - Fidgets, squirms in seat - Leaves seat, when being seated is expected - Run about, inappropriately - Unable to remain quiet while at play or leisure - On-the-go; motor driven - May talk excessively - Blurts out, cannot wait for turn - Interrupts, intrudes <u>Symptoms causing impairments were present before 12 years old</u> Impairment from the symptoms are present in two or more settings (school & home) The symptoms do not occur exclusively during the course of another mental disorder	≥6 symptoms for ≥6 months	If both (1) and (2) are met for 6 months, this is <u>ADHD, Combined Presentation</u> If (1) is met for 6 months, this is <u>ADHD, Predominately Inattentive</u> If (2) is met for 6 months, this is <u>ADHD, Predominately Hyperactive-Impulsive</u> Severity is specified as: - Mild - Moderate - Severe	• Unspecified

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Paranoid Personality Disorder (PPD)	<u>Suspicion that other are being exploitative or deceptive</u> Unjustified doubts about loyalty/trustworthiness Reluctance to confide Reads hidden threatening meaning into remarks Reacts angrily to perceived insults (What do you mean have a good day?!)	Unspecified	Cluster A (odd-eccentric) Theme is Distrust & Suspiciousness Onset is gradual and typically presents in childhood	• CBT • Medication as adjunctive treatment
Schizoid Personality Disorder	<u>Lacks desire for close relationships/intimacy</u> Lacks close friends Prefers solitary activities Derives pleasure in few activities Indifference to praise/criticism Emotional coldness, detachment or <u>flat affect</u>	Unspecified	Cluster A (odd-eccentric) Theme is Interpersonal Detachment	• CBT • Medication as adjunctive treatment
Schizotypal Personality Disorder	Social and interpersonal deficits with acute discomfort for close relationships <u>Odd speech/thinking with beliefs in paranormal</u> Ideas of reference Odd appearance/behavior Suspiciousness (often results in social anxiety)	Unspecified	Cluster A (odd-eccentric) Theme is Eccentric <u>Considered a premorbid personality of schizophrenia</u>	• CBT • Medication as adjunctive treatment
Histrionic Personality Disorder	Needs to be center of attention (uses physical appearance) Inappropriately provocative behavior Shallow and rapidly shifting emotions (exaggerated) <u>Impressionistic speech but lacks detail</u> Exaggerated emotional expression Considers relationships intimate	Unspecified	Cluster B (dramatic, emotional, erratic) Theme is Excessive Emotionality	• CBT • Medication as adjunctive treatment

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Borderline Personality Disorder	<u>Unstable intense relationships with alternating idealization and devaluation (splitting)</u> Frantically avoids abandonment Intense uncontrolled anger Marked reactivity of mood Chronic feelings of emptiness Unstable self image Self-damaging impulsivity <u>Suicidal gestures/self-mutilation</u> Transient stress-related dissociative/paranoid symptoms	Unspecified	Cluster B (dramatic, emotional, erratic) Theme is Instability	• CBT • Medication as adjunctive treatment
Narcissistic Personality Disorder (NPD)	Grandiose sense of self-importance Preoccupied with success Requires admiration Sense of entitlement Interpersonally exploitative Lacks empathy with haughty attitudes	Unspecified	Cluster B (dramatic, emotional, erratic) Theme is Grandiosity A “narcissistic injury” (blow or threat to image) could result in rage	• CBT • Medication as adjunctive treatment
Antisocial Personality Disorder (ASPD)	Repeated unlawful acts Deceitfulness & Lack of Remorse Irritability and aggressiveness Reckless disregard for safety of self or other Irresponsibility (employment or financial) <u>Must be at least 18 and have had a history of Conduct Disorder before 15 years old</u>	Unspecified	Cluster B (dramatic, emotional, erratic) Theme is Pervasive Disregard for Other’s Rights Associated with “psychopath”	• CBT • Medication as adjunctive treatment

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Avoidant Personality Disorder	Avoids interpersonal and occupational activities for <u>fear of criticism</u> Unwilling to start relationship unless certain of being liked Restraint within relationship for fear of ridicule Reluctance to engage in new activities <u>Negative self-image</u>	Unspecified	Cluster C (anxious/fearful) Theme is Inferiority Complex Unlike schizoid, <u>those with Avoidant Personality Disorder want a relationship</u>	• CBT • Medication as adjunctive treatment
Dependent Personality Disorder	Indecisive/Difficult Disagreeing Others must take responsibility for life Difficulty initiating due to low confidence Feels helpless when alone Urgently seeks another relationship if one ends	Unspecified	Cluster C (anxious/fearful) Theme is Excessive need to be cared for	• CBT • Medication as adjunctive treatment
Obsessive-Compulsive Personality Disorder (OCPD)	Preoccupation with details, rules, lists until point of activity is lost Perfectionism interferes with task completion Excessive devotion to work Over conscientious, scrupulous, rigid and stubborn Reluctance to delegate or to work with others	Unspecified	Cluster C (anxious/fearful) Theme is Inflexible perfectionism & control Type "A" personalities <u>Associated with Coronary Artery Disease</u>	• CBT • Medication as adjunctive treatment
Oppositional Defiant Disorder (ODD)	Behaviors/emotions characterized by: - Angry/Irritable Mood - Argumentative/Defiant Behavior (<u>deliberately annoys</u>) - Vindictiveness (wanting to get revenge)	Unspecified	Basically a brat kid Behaviors do <u>not</u> result in a serious violation of the rights of others	• Anger Management • Communication Skills • Parental Training

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Conduct Disorder (CD)	Repeated serious violation of rights/societal norms Multiple symptoms occurring in or across any of the categories: - Aggressive conduct: bullies, uses weapons, cruelty to people/animals, rape, stolen with confrontation - Deliberate property destruction: by fire or other means - Deceit or theft: broken in, lies, stolen without confronting - Serious violation of rules: breaking curfew running away, truancy	Unspecified	Subtypes: - Childhood-Onset: < 10 years old, characterized by <u>aggressive conduct</u> - Adolescent-Onset: ≥ 10 years old, characterized by <u>serious violation of rules</u> <u>Precursor for ASPD</u>	• Anger Management • Communication Skills • Parental Training
Intermittent Explosive Disorder (IED)	Recurrent outbursts representing a failure to control aggressive impulses as manifested by either: - <u>Verbal aggression or non-damaging physical aggression</u> occurring frequently - <u>Damaging physical aggression</u> occurring infrequently Aggression is: - Disproportionate to stressor - Impulsive and/or anger based - Not explained by other causes/disorder	Unspecified	Described as “brief spells” <i>Those with IED are usually not aggressive and don’t violate rights in between explosive episodes</i>	• Anger Management
Kleptomania	Recurrent irresistible stealing of <u>unneeded</u> objects Increasing tension before stealing Pleasure, gratification, or relief when stealing No other cause or motivating factor for theft	Unspecified	Stolen items are unwanted and typically of low value Focus is the process, not the goal <u>Usually due to low serotonin (prefrontal impulse control) & high dopamine (reward sensation) during act</u>	• Behavioral Therapy - Stimulus Control - Aversion Therapy - Exposure & Response Prevention • Behavioral Therapy • Medication (SSRI)

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Pyromania	Multiple episodes of deliberate fire setting with preceding tension or emotional arousal Fascination with fire and fire paraphernalia Pleasure, gratification, or relief when setting fires or witnessing/participating in the aftermath No other cause or motivating factor	Unspecified	Different from arsonist who has motivation for setting fire (money, political, homicidal) <u>Usually due to low serotonin (prefrontal impulse control) & high dopamine (reward sensation) during act</u>	• Behavioral Therapy - Stimulus Control - Aversion Therapy - Exposure & Response Prevention • Behavioral Therapy • Medication (SSRI)
Insomnia Disorder	Difficulty initiating or maintaining sleep for ≥ 3 months	≥ 3 months	Usually due to classical conditioning where the bed is associated with other habits besides sleeping	• Stimulus Control Technique • Sleep Hygiene Methods • Pharmacological - Sedatives (only short term due to decreased slow wave sleep and REM & tolerance) - Benzodiazepines (fewer side effects)
Hypersomnolence Disorder	Excessive sleepiness despite sufficient sleep (at least 7 hours for ≥ 3 months)	≥ 3 months	9.5 hours sleep episodes Unrefreshing naps <u>Normal PSG</u>	• Stimulants
Narcolepsy	PSG (short REM & sleep onset latency) Recurrent irresistible sleep occurring within the same day, several times per week for ≥ 3 months At least <u>one</u> of the following: - Cataplexy (loss of muscle strength) - Hypocretin (orexin) deficiency (spinal tap!) - Characteristic PSG abnormalities : -Nocturnal PSG (short REM latency) -Daytime PSG (short REM <u>&</u> sleep onset latency)	≥ 3 months	Other REM-related behaviors may be present but are NOT diagnostic criteria: - Sleep paralysis - Hypnagogic - Hypnopompic	• Polytherapy - Stimulants - Antidepressants (for cataplexy) • Monotherapy (Xyrem) - Very restricted - Also known as GHB which is illegal

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Obstructive Sleep Apnea Hypopnea (OSAH)	Multiples episodes of breathing cessation/reduction occur per night <u>due to an upper airway obstruction</u> Obstruction usually occurs when the soft tissue in the back of the throat collapses during sleep <u>PSG will show thoracic efforts</u>	Unspecified	Classically middle aged, overweight male who snores loud and intermittently Sleep duration may be adequate but quality is unrefreshing	• CPAP • Avoid back sleeping • Orthodontic Devices • Surgery
Central Sleep Apnea (CSA)	Multiple episodes of cessation of breathing per night <u>caused by CNS dysregulation of breathing</u> <u>PSG will NOT show thoracic efforts (dangerous)</u>	Unspecified	Can be idiopathic or from opioid use	• Varies depending on cause
Circadian Rhythm Sleep-Wake Disorder (CRSWD)	Excessive sleepiness or insomnia resulting from <u>a mismatch between a person's circadian sleep-wake pattern and the sleep-wake schedule required by the environment</u>	Unspecified	Fancy term for jet lag	• Strategic Phototherapy
Non-REM Sleep Arousal Disorder (N-RSAD)	Repeated episodes of incomplete awakening from sleep with either of the following: - Sleep Walking (relatively unresponsiveness/difficulty waking) - Sleep Terrors (intense fear/autonomic arousal) <u>Episode occur within the first 1/3 of sleep with amnesia of the dream or episode</u>	Unspecified	Subtypes: - Sleep Walking - Sleep Terrors	• Benzodiazepines to decrease slow wave sleep • Safe environment for sleep walker

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Nightmare Disorder	Extremely dysphoric dreams that typically involve threats to survival, security, or physical integrity <u>Characterized by awakening in the 2nd half of sleep period (during REM sleep), rapid alertness upon awakening, with good recall of the episode and dream</u>	Unspecified	Can recall episode next morning	• Antidepressants to decrease REM
REM Sleep Behavior Disorder (RBD)	<u>Vocalization and/or complex motor movements occur during REM sleep</u> Confirmed by PSG Cannot be induced by a substance	Unspecified	Typically action filled, violent dreams Immediately awake, oriented, and alert with detailed dream recall Can be associated with neurodegenerative disease (Parkinson's or Lewy Body Dementia)	• Clonazepam • Modification of sleep environment
Restless Leg Syndrome (RLS)	Urge to move legs in response to uncomfortable sensations with all the following features: - Occurs/worsens during inactivity - <u>Nocturnal worsening of symptoms</u> - Temporary relief from discomfort by moving <u>Patient is aware of symptoms and complains of insomnia</u>	Unspecified		• Anti-Parkinson's drugs to increase DA
Periodic Limb Movements (PLMs)	Repetitive muscle contractions during sleep, usually of the lower limb: - Associated with multiple sleep stage arousals - <u>Patient complains of daytime sleepiness but is unaware of the movements</u> - Electromyogram with PSG confirms diagnosis	Unspecified	PLMs differ from "sleep startles", which are muscle contractions that occur only upon falling asleep	• Anti-Parkinson's drugs to increase DA

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Gender Dysphoria (GD)	A marked incongruence between one's assigned (natal) gender and one's experienced This incongruence results in clinically significant distress or function impairment Symptoms endure at least 6 months	≥ 6 months	Strong and persistent beliefs Thought to result from altered androgen exposure - No androgen = female gender identity - Androgenized = male gender identity	• Transgender Living • Hormones Treatment • Sex Reassignment Surgery
Female Sexual Interest/Arousal Disorder	Problems with libido and/or physiological arousal	≥ 6 months	Sexual Dysfunction disorder	• Duel Sex Therapy • PLISSIT
Female Orgasmic Disorder	A delay in, the inability to, or having low-intensity orgasms	≥ 6 months	Sexual Dysfunction disorder	• Duel Sex Therapy • PLISSIT
Genito-Pelvic Pain/Penetration	Difficulties with <u>vaginal penetration (specifically)</u> or vulvovaginal/pelvic pain during intercourse/penetration attempts	≥ 6 months	Sexual Dysfunction disorder	• Duel Sex Therapy • PLISSIT
Male Hypoactive Sexual Desire Disorder	Problems with low libido	≥ 6 months	Sexual Dysfunction disorder	• Duel Sex Therapy • PLISSIT
Erectile Disorder	Problems maintaining or attaining an erection	≥ 6 months	Sexual Dysfunction disorder Can be tested with Snap Gauge	• Duel Sex Therapy • PLISSIT
Premature Ejaculation	Ejaculation occurring during partnered activities ≈1 minute following vaginal penetration	≥ 6 months	Sexual Dysfunction disorder	• Squeeze • Star and Stop • SSRIs
Delayed Ejaculation	Marked delay in, or absence of, ejaculation	≥ 6 months	Sexual Dysfunction disorder	• Duel Sex Therapy • PLISSIT

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Exhibitionism	Sexual arousal from exposing genitals to an unsuspecting strangers	≥ 6 months	"Flashers"	• Aversion Therapy • Covert Sensitization • SSRI's (impulse control) • Anti-androgens (Depo-Provera)
Voyeurism	Sexual arousal from observing unsuspecting person either naked, in the process of disrobing, or engaging in sexual activity Person must be at least 18 years	≥ 6 months	If under 18, the person does not have a paraphilia, this is considered curiosity	• Aversion Therapy • Covert Sensitization • SSRI's (impulse control) • Anti-androgens (Depo-Provera)
Fetishism	Sexual arousal from nonliving objects (studded leather) or a highly-specific sexual focus on a non-genital body part (foot)	≥ 6 months	Devices designed for tactile genital stimulation do not count as a fetish	• Aversion Therapy • Covert Sensitization • SSRI's (impulse control)
Transvestism	Sexual arousal by <u>cross-dressing</u> (wearing the opposite sex's clothing)	≥ 6 months	"Cross-dressers"	• Aversion Therapy • Covert Sensitization • SSRI's (impulse control)
Frotteurism	Sexual arousal from <u>touching or rubbing against a non-consenting person</u>	≥ 6 months	"Frot" means to touch or rub genitalia	• Aversion Therapy • Covert Sensitization • SSRI's (impulse control) • Anti-androgens (Depo-Provera)
Sexual Sadism	Sexual arousal from <u>causing</u> psychological/physical suffering of another person	≥ 6 months	"The person with the whip"	• Aversion Therapy • Covert Sensitization • SSRI's (impulse control) • Anti-androgens (Depo-Provera)
Sexual Masochism	Sexual arousal from <u>receiving</u> psychological/physical suffering	≥ 6 months	"The person receiving the spanking"	• Aversion Therapy • Covert Sensitization • SSRI's (impulse control) • Anti-androgens (Depo-Provera)
Pedophilia	Sexual arousal by a prepubescent child (usually 13 or younger) <u>The perpetrator must be at least 16 years old AND the child must at least 5 years older than the child</u>	≥ 6 months	Sex offenders are legally required to register (Megan's Law)	• Aversion Therapy • Covert Sensitization • SSRI's (impulse control) • Anti-androgens (Depo-Provera)

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Substance Intoxication	The development of a <i>reversible</i> substance-specific syndrome due to recent ingestion of a drug The syndrome is due to drug effects on the CNS and <u>causes significant maladaptive behavioral or psychological changes</u>	Unspecified	<u>Alcohol Intoxication</u> specifically requires a significant maladaptive behavioral change PLUS ≥ 1 of the following: - Slurred speech - Incoordination - Unsteady gait - Nystagmus - Impaired cognition or memory - Stupor or coma	• Drug Rehabilitation (acute phase)
Substance Withdrawal	The development of a substance-specific syndrome <u>following cessation of a substance after heavy/prolonged substance use</u> The syndrome causes significant distress or impairment in functioning	Unspecified	<u>Alcohol Withdrawal</u> specifically requires ≥ 2 of the following: - ANS hyperactivity - Hand tremor - Insomnia - Nausea - Hallucinations - Psychomotor agitation - Anxiety - Seizures Symptoms usually appear within 72 hours after reduction of drug use and may last several weeks	• CBT with Drug Rehabilitation

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Substance Use Disorder	A cluster of cognitive, behavioral, and physiological symptoms indicating that the individual <u>continues to use a substance despite significant substance-related problems</u> A maladaptive pattern of substance use as manifested by ≥ 2 of the following 11 symptoms in a 12 month period: - <u>Impaired Control</u> 1. Taken in a larger amount than intended 2. Persistent unsuccessful attempts to cut back 3. Time consuming (obtaining, using, recovering) 4. Cravings or strong desire for substance - <u>Social Impairment</u> 5. Failure to fulfill major role obligations 6. Social/interpersonal problems related to drug 7. Reduction of important activities - <u>Risky Use</u> 8. Use in physically hazardous situations 9. Use despite having physical/psychological problem to substance - <u>Pharmacological Criteria</u> 10. Tolerance (↓effect of dose due to repeated use) 11. Withdrawal syndrome upon cessation	Unspecified	If tolerance and withdrawal are the only two symptoms after appropriate use of a prescribed drug, <u>then don't diagnose SUD</u> Severity of SUD is based on the # of symptoms out of 11 Caffeine use does NOT qualify as a SUD	• CBT with Drug Rehabilitation
Delirium	Involves: - A disturbance in <u>awareness and attention</u> - An additional disturbance in a cognitive domain - Sudden onset of symptoms that typically fluctuate during the day - Evidence for a direct physiological cause	Unspecified	Core deficits in the Reticular Activating System and its ascending connections Core deficits in central cholinergic functioning	• Treat underlying medical condition • Utilize environmental supportive measures • Manage associated symptoms by: - Antipsychotics to treat associated symptoms - Benzodiazepines to treat delirium caused by alcohol withdrawal

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Amnesia	A significant <u>acquired memory deficit</u> caused by a medical condition or the effect of a substance	Unspecified	Typically caused by damage to the hippocampus If lengthy retrograde amnesia, there is often <u>a temporal gradient to the memory loss with recent long-term memories more impaired than remotes long-term memories</u>	• Treat underlying cause • Cognitive Rehabilitation - Restoration of Function - Compensation - External Strategies - Internal Strategies
Dementia	A dementia refers to: <u>Multiple and severe cognitive impairments</u> without impaired consciousness Progressive and irreversibly and most commonly occurs in the elderly	Unspecified	Not to be confused with Mild Cognitive Impairment which is just cognitive decline that does not affect activities of daily living	• Unspecified
Alzheimer's Dementia (AD)	<u>Significant memory impairment plus impairment in at least 1 other cognitive domain</u> A gradual onset with steadily progressive decline <u>Early stages:</u> memory deficits and anomia <u>Middle stages:</u> further memory and language decline, visuospatial deficits, agnosia, mood changes, psychosis <u>Late stages:</u> global aphasia, motor dysfunction, death from opportunistic infections Multiples causes that need to be ruled out before diagnosis: - Vascular, frontotemporal degeneration, Lewy bodies, Parkinson's, Huntington's, Prions	Unspecified	Associated with: - Cortical atrophy - Hippocampal atrophy - Enlarged ventricles Neurochemical shows neurotransmitter deficiencies (<u>ACH in the nucleus basalis</u>) Neurofunction shows posterior hypometabolism Histology shows β-amyloid plaques and neurofibrillary triangles	• Cholinesterase Inhibitors (mild-moderate symptoms) - Donepezil (Aricept) - Galantamine (Razadyne) - Rivastigmine (Exelon) • NMDA Receptor Blocker (moderate-severe symptoms) - Memantine (Namenda) • Neuropsychiatric treatment

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Vascular Dementia	(Differential diagnosis of Alzheimer's Dementia) Dementia results from <u>multiple infarcts caused by cerebral vascular disease (CVD)</u>	Unspecified	Patient history typically has signs and symptoms of CVD Sudden onset with stepwise progression Headache and seizure onset are more common in early stages	• Treat underlying CVD to prevent further damage • Alzheimer's drugs for cognitive problems
Frontotemporal Dementia (FTD)	(Differential diagnosis of Alzheimer's Dementia) <u>Earlier age of onset (40s-50s)</u> Similar to AD but frontal signs are prominent early in the dementia (personality change)	Unspecified	Associated with frontal lobe atrophy and hypometabolism Histopathology changes are seen (<u>Pick bodies in Pick's Disease</u>)	• Unspecified
Lewy Body Dementia (LBD)	(Differential diagnosis of Alzheimer's Dementia) Classic triad of core clinical features: - Fluctuating cognition/alertness - Visual hallucinations (random, non-upsetting) - Mild parkinsonism (not tremors)	If dementia develops <u>first</u> or within first 12 months	1-Year Rule - If dementia develops first or within first 12 months, this is LBD	• Treat hallucinations with "benign neglect" • L-Dopa is relatively ineffective in LBD and worsen psychosis
Parkinson's Disease Dementia (PDD)	(Differential diagnosis of Alzheimer's Dementia) Up to 50% of PD patients develop dementia with similar pathology to LBD	If dementia develops >12 months <u>after</u> well established PD	1-Year Rule - If dementia develops >12 months after well-established PD, this is PDD	• Unspecified
Huntington's Disease Dementia (HDD)	(Differential diagnosis of Alzheimer's Dementia) Dementia develops <u>after</u> the onset of choreoathetosis and psychiatric symptoms	Unspecified	Choreoathetosis is the occurrence of involuntary movements in a combination of chorea (irregular migrating contractions) and athetosis (twisting and writhing).	• Unspecified

<u>Disorder</u>	<u>Symptoms Required</u>	<u>Duration</u>	<u>Notes</u>	<u>Treatment</u>
Prion Disease	(Differential diagnosis of Alzheimer's Dementia) Dementia <u>progresses rapidly over a few months with death under a year</u>	Unspecified	Example is <i>Creutzfeldt-Jakob</i>	• Unspecified
Reversible Dementia	(Differential diagnosis of Alzheimer's Dementia)	Unspecified	Usually due to infections	• Treatment of infection
Pseudodementia	(Differential diagnosis of Alzheimer's Dementia) <u>Depressed elderly patients</u> often show memory and other cognitive disturbances that resemble a dementia	Unspecified	Also known as Dementia Syndrome of Depression	• Depression treatment
Benign Senescent Forgetfulness	(Differential diagnosis of Alzheimer's Dementia) Cognitive decline associated with normal aging	Unspecified	Determined by neuropsych testing whether current cognition reflects a pathological process	• Unspecified
Major NCD (Neuro Cognitive Disorder)	Significant decline in at least <u>one</u> cognitive domain: - Deficits interfere with independence in daily activities	Unspecified	Current DSM-5 term for dementia/amnesia Has multiple etiological subtypes Patients are often referred for neuropsychological testing, involving an <u>extensive</u> evaluation of a person's cognitive function	• Unspecified
Minor NCD (Neuro Cognitive Disorder)	Modest decline in at least <u>one</u> cognitive domain: - Deficits <u>DO NOT</u> interfere with independence in daily activities	Unspecified	Current DSM-5 term for dementia/amnesia Has multiple etiological subtypes Patients are often referred for neuropsychological testing, involving an <u>extensive</u> evaluation of a person's cognitive function	• Unspecified